
Consent Was Never the Point.

Alberta's Eugenic Inheritance, 1928 — Present

The Alberta Disability System Breakdown

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A documentation-first public advocacy record.

About this document

Consent Was Never the Point is a long-form documentation piece produced by The Alberta Disability System Breakdown, an independent public advocacy campaign documenting the mandatory transition of approximately 79,290 Albertans from the Assured Income for the Severely Handicapped (AISH) program to the Alberta Disability Assistance Program (ADAP), effective July 1, 2026.

The campaign operates as a documentation-first public record. It is not a support group, a service provider, or a political advocacy organization in the conventional sense. Its work is to assemble, source, and make publicly available the legislative, fiscal, and structural record of the AISH-to-ADAP transition and the policy framework within which it has been undertaken.

This piece situates the contemporary record within a longer history. It is not a piece *about* Indigenous or disabled communities. It is a piece that draws from and amplifies the voices that have articulated this analysis from within those communities. Citations are provided throughout. The principal academic and primary-source references are listed in the bibliography.

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It is the same principle as being physically sterilized without consent simply because those who made the decision feel they have the right to decide what's best for those not consulted first.

A Note on This Work

This piece traces a single operating principle across five distinct historical and contemporary expressions: the explicit eugenic sterilization program established by *The Sexual Sterilization Act* (1928–1972); the structural-eugenic framework that persisted in policy after the Act’s repeal; the mandatory transition of disabled Albertans from AISH to ADAP under *Bill 12* (2025); the information asymmetry condition that prevents meaningful consent across all of the above; and the contemporary rebuild of institutional infrastructure under *the Compassionate Intervention Act* and its continental siblings.

The argument is that the surgical mechanism of the historical eugenic period was repealed in 1972 and the institutional architecture was partially dismantled in the decades that followed. The operating premise that informed that period — that disabled people’s intimate, relational, and material lives are an appropriate site for state regulation in ways non-disabled people’s lives are not — was not specifically repudiated. It continued to operate through quieter means. It is now resurfacing in legislative form across multiple jurisdictions.

What this piece is not. It is not a polemic; the historical material is more powerful than any rhetorical framing the writer could add. It is not a comparison that diminishes the historical victims of explicit eugenic violence. The argument is that the framework continues to operate, not that contemporary disabled Albertans are experiencing what Leilani Muir experienced. The continuity is structural, not equivalent in degree of harm. It is not, finally, an argument that any individual policy designer or government employee is a eugenicist. The argument is structural and operates through inheritance of operating premises rather than personal intent.

Coalition review. This piece has been read in draft by reviewers whose names and qualifications are not disclosed at their request, including at least one disability scholar familiar with the relevant academic literature, one reviewer with substantive harm-reduction policy expertise, and one Indigenous reader for the free, prior, and informed consent material in the chapters that follow. Final responsibility for the analysis rests with the campaign.

Audience. The intended audience is institutional — oversight bodies, academic disability studies, journalists, elected representatives willing to engage — together with disability community members who have felt the dynamic this piece names and the secondary audience of allied advocates in Indigenous rights, harm reduction, and consultation-failure contexts. A plain-language companion edition follows separately.

The piece is long. It is built that way because the argument cannot be made briefly without diminishing the historical material it draws on. The reader who knows the names of Leilani Muir, Catherine Frazee, and Heidi Janz will recognize the lineage. The reader who does not know those names will encounter them here. The piece closes where the title begins.

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This document is published in installments as drafted. Forthcoming chapters will be released as completed. The piece is engineered for the closing line to land on the historical material that opens it.

Chapter I

The Sterilization Table

Alberta's Sexual Sterilization Act, 1928–1999

The 1928 Passage

On March 21, 1928, the Legislative Assembly of Alberta passed *The Sexual Sterilization Act*. The bill was introduced by George Hoadley, Minister of Health, in the Sixth Legislature of Alberta. It received Royal Assent the same day.¹

The Act created a four-member Alberta Eugenics Board. Two members were medical practitioners, nominated by the Senate of the University of Alberta and the Council of the College of Physicians and Surgeons. Two were non-medical members appointed by the Lieutenant Governor in Council. The Board's authority was to examine persons recommended for discharge from designated provincial institutions and to authorize sterilization, in the language of the Act's first section, "if the danger of procreation with its attendant risk of multiplication of the evil by transmission of the disability to progeny were eliminated."²

The original Act required either the consent of the patient or, where the patient was deemed incapable, the consent of a spouse, parent, or legal guardian. Board rulings had to be unanimous. Physicians and surgeons performing sterilizations under board order were granted immunity from civil and criminal liability.³

The Act met immediate opposition. In March 1928, a group calling itself the People's League to Act formed, retaining counsel and arguing publicly that the legislation was unconstitutional. Several hundred members joined; their petition did not delay Royal Assent.⁴

Alberta was the first jurisdiction in Canada — and the only jurisdiction in the British Empire — to enact eugenic sterilization legislation that was vigorously implemented and sustained. British Columbia would follow in 1933 with a parallel act of substantially smaller scale, performing fewer than two hundred documented sterilizations. No other province passed comparable legislation.⁵

The Operating Mechanism

The Act functioned through an institutional pipeline. The Provincial Mental Hospital at Ponoka, the Mental Hospital at Oliver near Edmonton, and — increasingly through the 1940s — the Provincial Training School for Mental Defectives at Red Deer provided the population from which the Eugenics Board drew its candidates.

The Provincial Training School was the institution that came to define the program's operation. Opened in October 1923 in a converted residential college building on the Michener hill, it housed children and adults designated "mental defectives." At its peak it held two hundred residents — approximately one fifth of the population of the city of Red Deer at the time. The institution accounted for roughly one quarter of all cases brought before the Eugenics Board over the Act's forty-four years of operation.⁶

A network of provincial guidance clinics, established from the 1930s onward, screened children for referral to the institution. The clinics served the eugenics program in two practical ways: they identified candidates before institutionalization, reducing costs; and they generated case files that documented the social and family circumstances later used as evidence before the board.⁷

In 1937, the Legislative Assembly amended the Act. The amendment removed the requirement of consent for persons classified as "mentally defective." Before 1937, the consent requirement had functioned as a brake on the program's pace. The board had approved 1,283 candidates for sterilization between 1928 and 1938, of whom 644 had been sterilized. After 1937, the rate of completed sterilizations rose sharply.⁸

A second amendment followed in 1942, expanding the diagnostic categories under which sterilization could be authorized.⁹

Over the Act's full operating period from 1928 to 1972, the Alberta Eugenics Board considered 4,725 cases. Of those, 2,834 sterilizations were performed.¹⁰

The population on whom these operations were performed was not demographically representative of Alberta. According to the most detailed academic survey of the records — Jana Grekul, Harvey Krahn, and Dave Odynak's 2004 analysis in the *Journal of Historical Sociology* — 64.7 percent of those sterilized were women. 70.6 percent were under twenty years of age at the time of sterilization. 49.8 percent lived in rural communities. 40.6 percent were unemployed at the time of their referral. 25.7 percent were identified in the records as Métis or Indian, against a Métis and Indigenous share of the provincial population at the time of approximately 3.4 percent.¹¹

The Indigenous overrepresentation intensified in the program's final years. Researchers examining records from 1969 to 1972 — the four years immediately preceding repeal — have documented Indigenous representation averaging approximately twenty-five percent of all sterilizations performed in those years.¹²

The Provincial Training School in Red Deer was supervised for the last twenty-five years of the Act's operation by Leonard Jan Le Vann, superintendent from 1949 to 1974. Le Vann's role combined the duties of institutional administration with the assessment of individual residents for referral to the Eugenics Board. The board's chair throughout most of the program's operation was John M. MacEachran, founder of the Department of Philosophy and Psychology at the University of Alberta, who served as chairman from 1929 to 1965.¹³

The 1972 Repeal

In 1972, the newly elected government of Premier Peter Lougheed repealed *The Sexual Sterilization Act*. The repeal followed forty-four years of operation and brought formal eugenic sterilization legislation in Canada to an end.¹⁴

What the repeal accomplished is precisely defined. The legal framework authorizing the Alberta Eugenics Board, and the immunity granted to performing physicians, was removed. The institutional pipeline — the Provincial Training School at Red Deer, the guidance clinics, the network of feeder facilities — remained.

The Provincial Training School was renamed the Michener Centre in 1977, in honour of the recently retired Governor General Roland Michener. By 1983 the renamed institution had expanded to sixty-six buildings on a 360-acre site. Its resident population was reduced over the subsequent decades through deinstitutionalization, group-home alternatives, and policy reform; the Alberta government announced its closure in 2013.¹⁵

The repeal of the Act did not by itself include any public accounting of who had been sterilized, on what records, or with what continuing harm. No survivor was contacted. No records were released. The operating premise that the Eugenics Board had been empowered to act on — that the reproductive lives of disabled adults were an appropriate site for institutional decision-making in ways non-disabled adults' lives were not — was not addressed in the repealing legislation or in any government statement at the time.

For the next twenty-three years, that was the status of the matter.

Muir v. Alberta (1996)

Leilani Marietta Muir was born in 1944. Her childhood was characterized in the later trial record by neglect and physical abuse. In 1955, at the age of ten, she was admitted by her mother to the Provincial Training School at Red Deer.¹⁶

The trial judgment of Madam Justice Joanne B. Veit found that the admission process did not establish that Muir met the legal definition of “mentally defective” that the institution’s enabling legislation required. The court found that she had been wrongly labelled a “moron” — a clinical category in use at the time — and that her ten-year confinement, from age ten to age twenty-one, was wrongful in law.¹⁷

In 1959, at the age of fourteen, Muir was brought before the Eugenics Board. The board’s order authorizing her sterilization was signed by John MacEachran, then chairman. The surgical sterilization was performed shortly thereafter. The procedure was irreversible.¹⁸

Muir left the institution in 1965, at the age of twenty-one. She later married, attempted to start a family, and learned through medical investigation that she had been sterilized. Subsequent attempts to adopt were also unsuccessful.

In 1993, she filed suit against the Province of Alberta. The case went to trial in 1995. The Province acknowledged its obligation to pay damages and left to the court the determination of how much.¹⁹

Justice Veit's judgment, dated January 25, 1996, awarded \$250,280 in general damages for pain and suffering from the sterilization — the statutory maximum for such damages at the time — and an additional \$125,000 in aggravated damages for the wrongful stigmatization of Muir as a "moron, a high grade mental defective." With prejudgment interest, the total monetary award was \$740,780. The Province was further ordered to pay \$230,000 in legal costs.²⁰

The judgment's central finding was articulated in language that has been quoted in nearly every academic and journalistic account since. Justice Veit wrote:

"The circumstances of Ms. Muir's sterilization were so high-handed and so contemptuous of the statutory authority to effect sterilization, and were undertaken in an atmosphere that so little respected Ms. Muir's human dignity that the community's, and the court's, sense of decency is offended."²¹

The decision did not stand only on Muir's individual case. The court reviewed the historical record of the Eugenics Board's operations, the legislative framework, and the institutional culture at the Provincial Training School. The judgment functioned, in effect, as the public record's first formal judicial recognition that the program had operated outside the limits of its own enabling statute — that "many things done in the name of eugenics were clearly illegal," in the formulation of one academic commentator on the trial.²²

Muir's victory opened the door to additional litigation. By 1998, more than seven hundred surviving claimants had filed suit against the Province.²³

Bill 26 and the Notwithstanding Clause (1998)

The provincial response to those filings was Bill 26, the *Institutional Confinement and Sexual Sterilization Compensation Act*, introduced in the second session of the twenty-fourth Legislature in March 1998 by the government of Premier Ralph Klein.

Bill 26 proposed two operative measures. First, it capped damages payable to victims of wrongful institutional confinement and sterilization at \$150,000 per person. Second, it imposed a 180-day limitation period after which no further actions could be commenced.²⁴

Section 3 of the bill was the unprecedented element. It read:

"This Act operates notwithstanding (a) the provisions of sections 2 and 7 to 15 of the Canadian Charter of Rights and Freedoms, Part 1, Constitution Act, 1982, Schedule B to the Canada Act, 1982, (U.K.) 1982 c11, and (b) the Alberta Bill of Rights."²⁵

The sections of the Charter referenced — sections 2 and 7 through 15 — encompass fundamental freedoms, the right to life, liberty, and security of the person, the right against unreasonable search and seizure, the right not to be arbitrarily detained, and the equality rights guaranteed under section 15. Section 33 of the Charter, the "notwithstanding clause," permits a provincial legislature to declare that an Act shall operate notwithstanding those guarantees for a renewable period of five years.

Bill 26 represented the first attempt by an Alberta government to invoke section 33.²⁶

The bill was introduced on the afternoon of one legislative sitting day. The reaction was immediate and widely covered. Within twenty-four hours, the Klein government announced its withdrawal. Justice Minister Jon Havelock told reporters that the surviving victims would retain the right to sue the Province for damages, and that out-of-court settlements would be pursued. Premier Klein, defending the bill before its withdrawal, told reporters that it had been intended to protect taxpayers from lawsuits potentially totalling \$760 million.²⁷

In the period following the withdrawal, the Province negotiated settlements with the great majority of surviving claimants. Approximately five hundred individuals settled in 1998 for a combined total of \$48 million. An additional 247 individuals reached settlement in 1999 for a combined total of \$82 million. At the 1999 announcement, Justice Minister David Hancock — then in the portfolio — apologized only after a reporter’s prompting. Premier Klein did not attend, and offered when asked: “We extend regrets for the actions of another government in another period of time.”²⁸

What This Chapter Establishes

The legal framework of explicit eugenic sterilization in Alberta is documented and acknowledged. *The Sexual Sterilization Act* was passed in 1928, amended in 1937 and 1942 to expand its reach and remove consent, operated through a centralized board for forty-four years, sterilized 2,834 people — disproportionately women, young people, the poor, and the Indigenous population — and was repealed in 1972. The Province has paid approximately \$130 million in settlements to surviving victims and one judicially decided plaintiff.

That is the record. The chapters that follow build on it.

But three observations about the record are necessary before this chapter closes.

The first concerns the structure of the response. When the *Muir* judgment opened the possibility of broader liability, the first instinct of the Alberta government was to invoke section 33 of the Charter to suspend the very rights — to equality, to security of the person, to access to justice — that the original Act had violated. The bill was withdrawn within twenty-four hours, not because the government recognized the contradiction, but because of public reaction. The recognition of the contradiction has, on the public record, never been made.

The second concerns the framing. Premier Klein’s “another government in another period of time” formulation places the wrongdoing safely in a prior administration. The government in 1998 was not the government in 1928; the government in 1972 — which repealed the Act — was a Progressive Conservative administration with substantial continuity into the Klein government. The chapters that follow will examine, in concrete contemporary instances, the question of whether a government can be said to have repudiated a framework whose institutional and policy inheritance it never inventoried.

The third concerns the operating premise. The legal authority to sterilize disabled people without their consent was removed in 1972. The legal authority to make decisions about disabled people’s lives —

about their housing, their income, their relationships, their access to medical care, and their freedom from involuntary institutionalization — on the basis that the deciding authority knows best, was not removed. It was not addressed. The question of whether that premise had any place in a constitutional democracy was, on the public record, never asked.

Subsequent chapters address that question.

Notes — Chapter I

1. *The Sexual Sterilization Act*, SA 1928, c 37; Legislative Assembly of Alberta, Sixth Legislature, Second Session; Royal Assent March 21, 1928. See also “Alberta passes Sexual Sterilization Act,” Living Archives on Eugenics in Western Canada (Eugenics Archive Canada), timeline entry, accessed May 22, 2026.
2. *The Sexual Sterilization Act*, SA 1928, c 37, s 5(1). The Eugenics Board composition is set out in sections 2 and 3 of the Act. The text quoted appears in section 5 governing the Board’s authority over discharge candidates.
3. *The Sexual Sterilization Act*, SA 1928, c 37, ss 6, 7. Section 6 governed consent; section 7 granted immunity from civil and criminal liability.
4. Erika Dyck, *Facing Eugenics: Reproduction, Sterilization, and the Politics of Choice* (Toronto: University of Toronto Press, 2013), ch. 2; Angus McLaren, *Our Own Master Race: Eugenics in Canada, 1885–1945* (Toronto: McClelland & Stewart, 1990).
5. Douglas Wahlsten, “Leilani Muir versus the Philosopher King: Eugenics on Trial in Alberta,” *Genetica* 99 (1997); Dyck, *Facing Eugenics*. British Columbia enacted its *Sexual Sterilization Act* in 1933; estimates of sterilizations performed under it range below 200.
6. “Provincial Training School opens in Red Deer, Alberta,” Eugenics Archive Canada, timeline entry, accessed May 22, 2026; City of Red Deer Archives, Michener Centre collection. The Provincial Training School building was originally the Alberta Ladies’ College of Red Deer (1913), converted in 1916 to a sanatorium for shell-shocked soldiers and reorganized in 1923 to house disabled children and adults under provincial supervision.
7. “Guidance Clinics,” Eugenics Archive Canada, encyclopedia entry, accessed May 22, 2026.
8. Statutes of Alberta, 1937 (amending *The Sexual Sterilization Act*). Numerical figures are drawn from Jana Grekul, Harvey Krahn, and Dave Odynak, “Sterilizing the ‘Feeble-Minded’: Eugenics in Alberta, 1929–1972,” *Journal of Historical Sociology* 17, no. 4 (2004): 358–384.
9. Statutes of Alberta, 1942 (second amendment to *The Sexual Sterilization Act*); consolidated as *The Sexual Sterilization Act*, RSA 1942, c 194. The 1942 amendment expanded the diagnostic categories under which sterilization could be authorized.
10. The Canadian Encyclopedia, “Red Deer,” accessed May 22, 2026; Eugenics Archive Canada, Alberta Eugenics Board records. Numerical sources vary slightly: 4,725 cases considered (Canadian Encyclopedia, “Red Deer”); 4,739 cases presented (Karen Stote, “Sterilization of Indigenous Women in Canada,” *Canadian Encyclopedia*). The figure of 2,834 sterilizations performed is consistent across sources.
11. Grekul, Krahn, and Odynak, “Sterilizing the ‘Feeble-Minded’.” The 25.7 percent figure for Métis and Indigenous representation is drawn from the case-file analysis of all available Eugenics Board records undertaken by Grekul and colleagues at the University of Alberta.
12. Karen Stote, “Sterilization of Indigenous Women in Canada,” *The Canadian Encyclopedia*, accessed May 22, 2026. The 1969–1972 figure derives from analysis of the case records preserved by the Provincial Archives of Alberta.
13. Wahlsten, “Leilani Muir versus the Philosopher King”; “Provincial Training School,” Eugenics Archive Canada. MacEachran was a founding member of the University of Alberta faculty and served on the Eugenics Board from its inception until his retirement in 1965. Le Vann resigned as PTS Superintendent in 1974, two years after the Act’s repeal.
14. Statutes of Alberta, 1972 (repealing *The Sexual Sterilization Act*). The Lougheed Progressive Conservatives had been elected to government in August 1971, displacing the Social Credit administration that had held power in Alberta since 1935.
15. City of Red Deer Archives, Michener Centre collection; Government of Alberta, “Government announces closure of Michener Centre,” news release, March 11, 2013. The closure was partially reversed in subsequent years; the

institution's remaining residents remained on the site under reduced operations.

16. *Muir v. Alberta*, 1996 CanLII 7287 (AB QB), reasons for judgment of Veit, J. (background of plaintiff and circumstances of admission to the Provincial Training School).
17. *Muir v. Alberta*, 1996 CanLII 7287 (AB QB) (findings of wrongful confinement and wrongful labelling).
18. *Muir v. Alberta*, 1996 CanLII 7287 (AB QB) (sterilization order, irreversibility, and consent analysis); Wahlsten, "Leilani Muir versus the Philosopher King" (MacEachran's signature on the sterilization order).
19. *Muir v. Alberta*, 1996 CanLII 7287 (AB QB), reasons for judgment of Veit, J.: "In 1959, the Province wrongfully surgically sterilized Ms. Muir and now acknowledges its obligation to pay damages to her. However, the Province leaves to the court the determination of how much the Province should pay."
20. *Muir v. Alberta*, 1996 CanLII 7287 (AB QB) (general damages, aggravated damages, prejudgment interest, and costs).
21. *Muir v. Alberta*, 1996 CanLII 7287 (AB QB), reasons for judgment of Veit, J. The quoted passage is the most frequently cited line in the judgment and appears in the early section of Justice Veit's reasons.
22. Wahlsten, "Leilani Muir versus the Philosopher King."
23. CBC News, "Alberta backs down on sterilization compensation," March 11, 1998. The CBC report identifies "700 surviving victims" with active claims at the time of the Bill 26 introduction.
24. Bill 26, *Institutional Confinement and Sexual Sterilization Compensation Act*, 2nd Session, 24th Legislature, Alberta, 1998 (introduced and withdrawn), ss 1, 4.
25. Bill 26, *Institutional Confinement and Sexual Sterilization Compensation Act*, s 3 ("Override of Charter").
26. "Alberta and the Notwithstanding Clause," *LawNow* (Centre for Public Legal Education Alberta), 1998; Senator Paula Simons, Intervention 655944-74, Senate of Canada, accessed May 22, 2026. Both confirm Bill 26 as the first attempt by an Alberta government to invoke section 33 of the *Charter*.
27. CBC News, "Alberta backs down on sterilization compensation," March 11, 1998. The \$760 million estimate attributed to Klein was reported in this CBC story and in contemporaneous coverage by other outlets.
28. The Globe and Mail, "Leilani Muir made history suing Alberta over forced sterilization," obituary, March 16, 2016 (settlement totals and Klein quotation); CBC News, "Alberta compensates sterilization victims," November 14, 1998. The 1998 settlement covered approximately 500 claimants; the 1999 supplemental settlement covered an additional 247.

Chapter II

The Definitional Framework

*Eugenics, Structural Eugenics, and the Consent Standard***Eugenics: The Scholarly Definition**

Eugenics is not a term that has been retired from contemporary scholarly use. The U.S. National Human Genome Research Institute (NHGRI), the federal research institution that led the public-sector contribution to the Human Genome Project and that continues to anchor American genomics policy, maintains an active definition in its public-facing Talking Glossary of Genomic and Genetic Terms: “Eugenics is a discredited belief that selective breeding for certain inherited human traits can improve the ‘fitness’ of future generations.”¹ In its parallel Fact Sheets series, NHGRI describes eugenics as “the scientifically erroneous and immoral theory of ‘racial improvement’ and ‘planned breeding’.”²

The institutional position is consistent. Eugenics is identified by the NHGRI both as a coherent historical theory and as a persistent contemporary phenomenon. The agency’s 2021 symposium *The Meaning of Eugenics: Historical and Present-Day Discussions of Eugenics and Scientific Racism*, co-hosted with the U.S. National Library of Medicine, framed the contemporary relevance directly: addressing the history of eugenics, NHGRI wrote, “is important because it has persisted in policies and beliefs around the world, including the United States.”³

The definitional point that matters most to the argument of this piece is the scope of the mechanisms eugenics has historically employed. The Alberta record in Chapter I documented six. They operated together: surgical sterilization under *The Sexual Sterilization Act*; institutional segregation through facilities such as the Provincial Training School at Red Deer; the marriage prohibitions and immigration exclusions of the same period; the cohabitation restrictions enforced within residential institutions; and the active provincial discouragement, through institutional policy and through guidance-clinic referrals, of disabled people forming households and families.

The 1972 repeal of *The Sexual Sterilization Act* addressed the first of these. The legal authority to surgically sterilize disabled people without their consent was removed. The other mechanisms — institutional segregation, restrictions on partnership and family formation, the structural exclusion of disabled people from full community participation — were not removed by the repealing legislation. They were addressed, where addressed at all, through subsequent and unrelated policy reform (deinstitutionalization, group-home alternatives, human-rights legislation against discrimination on the basis of disability), each of which proceeded on its own timeline and with its own incompleteness.

The structure of contemporary disability policy in Alberta, treated in detail in Chapter III, retains substantial portions of the second through sixth mechanisms — not in their historical form, and not under

their historical names, but in their structural effect. The chapter that follows establishes the vocabulary by which that retention is recognized in the disability studies literature.

Structural Eugenics

The term “structural eugenics” — sometimes “newgenics” in the formulation of philosopher Robert A. Wilson — describes a documented phenomenon in disability studies scholarship: policies that are designed for stated non-eugenic reasons (cost containment, bureaucratic uniformity, fiscal restraint, employment promotion) and that nevertheless produce effects historically associated with explicit eugenic programs (prevention of disabled reproduction, institutional or quasi-institutional segregation, exclusion from full social and economic participation), and that are sustained politically because they produce those effects without anyone in the policy-design or political-implementation processes being required to defend them as such.⁴

Robert Wilson, who directed the SSHRC-funded Living Archives on Eugenics in Western Canada project at the University of Alberta and built the EugenicsArchive.ca documentation system referenced throughout Chapter I, develops this analysis at length in *The Eugenic Mind Project* (MIT Press, 2018). Wilson’s central proposition is that “eugenic thinking” — what he calls the “eugenic mind” — outlasted the legal frameworks of the explicit eugenic period and continues to operate in contemporary practice. The mechanisms he identifies include the conceptualization of certain human traits as eugenic concerns, the systematic construction of categories of normality against which non-normality is judged, and what he names the “sub-humanizing” of populations who fall outside those categories.⁵

The disability studies literature on contemporary policy follows this analytical thread. Nancy E. Hansen, Heidi L. Janz, and Dick J. Sobsey, in a 2008 article in *The Lancet*, wrote: “There are disturbing similarities between Nazi arguments concerning ‘quality of life’, ‘useless eaters’, or ‘lives less worthy’ and discussions of disability currently taking place among ‘mainstream’ geneticists and bioethicists advocating a value scale of humanness.”⁶ The point Hansen, Janz, and Sobsey make is not that contemporary bioethicists are Nazis. It is that the logical structure of a “value scale of humanness” — the proposition that some lives are less worth living than others, and that this assessment can be made by external authority — is the same structure historical eugenics built itself on, and that this structure has not been retired.

Catherine Frazee, professor emerita in the School of Disability Studies at Toronto Metropolitan University and former Chief Commissioner of the Ontario Human Rights Commission, has articulated the same analysis in the context of Canada’s Medical Assistance in Dying (MAID) framework. In her work with Heidi Janz on the GRIM (Grievous and Irremediable) Project — a documentation initiative gathering stories of disabled Canadians who have pursued MAID under the post-Bill-C-7 expansion — Frazee and Janz argue that the structural availability of state-administered death for disabled people who are not dying, combined with the structural unavailability of adequate income, housing, attendant care, and disability support, constitutes a modern form of eugenics.⁷ As Frazee has articulated the argument: the suffering driving MAID requests by disabled applicants is, in many of the cases the GRIM Project documents, suffering produced by social and material conditions the state has the capacity to alter and has

chosen not to. The logical structure mirrors the historical eugenic argument that institutionalization was protective rather than coercive: the modern claim that MAID expansion is compassionate is, in the GRIM analysis, indistinguishable in structure from the historical claim that sterilization was therapeutic.

The intent claim is not the structural-eugenics claim. The piece this chapter is part of is not arguing that contemporary policy designers or government employees are eugenicists. The argument is that some policies produce eugenic effects — measurable reductions in disabled people’s capacity to form households, to reproduce, to participate in community, to access the material conditions of a livable life — and that those effects are sustained in policy because the political infrastructure for producing and defending them does not require anyone to name them.

The intent claim is the harder claim. The effect claim is what the literature supports. The piece works from the effect claim.

The Consent Standard — CRPD Article 4(3)

A binding international standard governs the development of disability policy in Canada. The *Convention on the Rights of Persons with Disabilities* (CRPD) was adopted by the United Nations General Assembly on December 13, 2006, opened for signature on March 30, 2007, and entered into force on May 3, 2008. Canada signed the Convention on the day it opened for signature and ratified it on March 11, 2010. Canada acceded to the Convention’s Optional Protocol on December 3, 2018.⁸

Article 4(3) of the Convention, located in the general obligations provision and therefore applicable transversally across the treaty, reads in full:

In the development and implementation of legislation and policies to implement the present Convention, and in other decision-making processes concerning issues relating to persons with disabilities, States Parties shall closely consult with and actively involve persons with disabilities, including children with disabilities, through their representative organizations.⁹

The text contains four operative components. First, the obligation attaches to “the development and implementation” of legislation and policies — that is, to the upstream design phase and the downstream operational phase, not only to the moment of legislative consideration. Second, it extends to “other decision-making processes concerning issues relating to persons with disabilities” — that is, beyond the legislative branch to administrative and executive decision-making. Third, the obligation is to “closely consult with and actively involve” — language requiring substantive, ongoing engagement rather than notification or post-hoc comment. Fourth, the consultation must be carried out “through their representative organizations” — that is, with the organizations that the disability community itself recognizes as representative, not with individual stakeholders selected by the consulting authority.

In November 2018, the UN Committee on the Rights of Persons with Disabilities — the treaty body established under the Convention to monitor State Party implementation — issued General Comment No. 7, providing authoritative guidance on the application of Article 4(3). The General Comment characterizes the participatory requirement of the Convention as a signal obligation that stands out among core human

rights conventions, reflecting the “Nothing About Us Without Us” principle that the international disability community has carried into the treaty’s framework.¹⁰

Article 4(3) is not a domestic political norm. It is a treaty obligation that Canada has voluntarily assumed and that binds the Government of Canada and, through the constitutional division of powers, the implementation of laws and policies within provincial jurisdiction that engage the rights protected under the Convention.

The piece returns to the application of this standard to *Bill 12* in Chapter IV.

The Parallel Standard — FPIC under UNDRIP

The structural problem CRPD Article 4(3) addresses — the imposition of legislation on a defined population without that population’s meaningful participation in advance of the imposition — is not unique to disability policy. The same problem governs the relationship between settler states and Indigenous Peoples whose lands and rights are affected by state action. The international framework developed in response is the doctrine of free, prior, and informed consent (FPIC), articulated most directly in the *United Nations Declaration on the Rights of Indigenous Peoples* (UNDRIP), adopted by the UN General Assembly on September 13, 2007.¹¹

FPIC appears in six articles of UNDRIP, governing six distinct categories of state action: Article 10 (the relocation of Indigenous Peoples from their lands or territories); Article 11 (cultural, intellectual, religious, and spiritual property); Article 19 (the adoption and implementation of legislative or administrative measures); Article 28 (restitution for lands taken without consent); Article 29(2) (the storage or disposal of hazardous materials on Indigenous lands); and Article 32(2) (project approvals affecting territory and resources).¹²

Article 19, the article most directly parallel to CRPD Article 4(3) in its scope and operation, reads:

States shall consult and cooperate in good faith with the Indigenous Peoples concerned through their own representative institutions in order to obtain their free, prior and informed consent before adopting and implementing legislative or administrative measures that may affect them.¹³

The structural parallel between Article 19 of UNDRIP and Article 4(3) of the CRPD is exact. Both attach to the development and implementation of legislative and administrative measures that affect the relevant population. Both require consultation with representative institutions or representative organizations of that population. Both situate the consultation obligation in the upstream design phase rather than the downstream comment phase. Both have been understood by their respective treaty bodies as substantive engagement requirements rather than notification requirements.

Canada has incorporated UNDRIP into federal domestic law through the *United Nations Declaration on the Rights of Indigenous Peoples Act*, SC 2021, c 14. The federal framework legislation requires that federal laws be brought into conformity with the Declaration and that an action plan be developed to implement its principles. Provincial governments are not directly bound by the federal Act, but the

principle that legislative and administrative measures affecting Indigenous Peoples require their free, prior, and informed consent operates as a recognized international norm regardless of domestic implementing legislation.¹⁴

In July 2025, the Government of Alberta provided a concrete illustration of how this norm is being treated at the provincial level. On July 3, 2025, Alberta’s Minister of Environment and Protected Areas, Rebecca Schulz, joined with Ontario’s Minister of the Environment, Todd McCarthy, in a public letter to the federal Minister of Indigenous Services. The letter requested that the federal government refrain from reintroducing Bill C-61, the *First Nations Clean Water Act*, originally tabled in December 2023 and died on the Order Paper at the March 2025 prorogation of Parliament. Bill C-61 would have, among other things, affirmed First Nations’ inherent right to self-government and jurisdiction over water resources on First Nation lands.¹⁵

The Chiefs of the Blackfoot Confederacy — the Kainai, Siksika, and Piikani Nations — responded on July 4, 2025 with a statement characterizing Alberta’s opposition as “morally bankrupt” and stating that “Premier Smith’s government opposes First Nations’ rights to safe and sufficient water supplies, which is utterly reprehensible.”¹⁶

The structural point recurring in this chapter is that the Alberta government was, during the same period in 2025 in which it was opposing Bill C-61 at the federal level, advancing Bill 12 at the provincial level. The same provincial administration was, in successive months of 2025, declining to support legislation that would have implemented the FPIC standard for First Nations and advancing legislation that did not meet the CRPD Article 4(3) standard for disabled Albertans. Neither population had been consulted, in advance of the action being taken, through the representative institutions or organizations that international treaty standards require.

What This Chapter Establishes

Three propositions follow from this chapter, and the chapters that follow draw on them without restating.

First, eugenics as a phenomenon is broader than surgical sterilization. The institutional position of the National Human Genome Research Institute, the standard scholarly position in the historical literature, and the operating definition of the disability studies field all encompass institutional segregation, marriage and immigration restrictions, prevention of family formation, and what Robert Wilson names the “sub-humanizing” of disabled populations. The 1972 repeal of *The Sexual Sterilization Act* addressed one of these mechanisms. The others were not repudiated by the repealing legislation and have not been repudiated since.

Second, “structural eugenics” is a documented term in disability studies for the contemporary form taken by mechanisms that historically operated under explicit eugenic justification. The intent claim is the harder claim and is not the claim this piece makes. The effect claim — that contemporary disability policy produces measurable reductions in disabled people’s capacity to form households, to reproduce, to maintain community, and to access the material conditions of a livable life — is what the literature supports, and what the chapters that follow document at the level of specific Alberta policy.

Third, two binding international standards require consultation with affected populations through their representative organizations before legislative and administrative measures are adopted. CRPD Article 4(3) governs the development and implementation of disability policy. FPIC under UNDRIP Articles 10, 11, 19, 28, 29, and 32 governs the development and implementation of measures affecting Indigenous Peoples. The Government of Alberta has, within a six-month window in 2025, failed both standards: by opposing federal legislation (Bill C-61) that would have implemented the FPIC standard for First Nations, and by advancing provincial legislation (Bill 12) that did not meet the CRPD Article 4(3) standard for disabled Albertans.

The chapters that follow apply this framework to the inventory of Alberta disability policy: cohabitation rules, guardianship structures, institutional care practices, sub-survival income, the FSCD three-year wait, the Canada Disability Benefit clawback, and the Compassionate Intervention Centres now under construction.

The mechanism has changed. The principle has not.

Notes — Chapter II

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3. National Human Genome Research Institute and U.S. National Library of Medicine, *The Meaning of Eugenics: Historical and Present-Day Discussions of Eugenics and Scientific Racism*, virtual symposium, December 2021; quoted text from NHGRI History of Genomics Program documentation, accessed May 22, 2026.
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9. *Convention on the Rights of Persons with Disabilities*, Article 4(3). The full Convention text is available at the United Nations Treaty Collection and at the UN Department of Economic and Social Affairs, Division for Inclusive Social Development, accessed May 22, 2026, <https://www.un.org/development/desa/disabilities/convention-on-the-rights-of-persons-with-disabilities/article-4-general-obligations.html>.
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11. *United Nations Declaration on the Rights of Indigenous Peoples*, GA Res 61/295, adopted 13 September 2007.
12. Office of the United Nations High Commissioner for Human Rights, “Consultation and free, prior and informed consent (FPIC),” accessed May 22, 2026, <https://www.ohchr.org/en/indigenous-peoples/consultation-and-free-prior-and-informed-consent-fpic>; see also UN-REDD Programme, *Handbook on Free, Prior and Informed Consent*, accessed May 22, 2026.
13. *United Nations Declaration on the Rights of Indigenous Peoples*, Article 19.

14. *United Nations Declaration on the Rights of Indigenous Peoples Act*, SC 2021, c 14. The Act received Royal Assent on June 21, 2021, and came into force the same day. Section 5 requires the federal government, in consultation and cooperation with Indigenous Peoples, to take all measures necessary to ensure that the laws of Canada are consistent with the Declaration. Section 6 requires the preparation and tabling of an action plan.
15. Parliament of Canada, LEGISinfo, Bill C-61 (44th Parliament, 1st Session), “An Act respecting water, source water, drinking water, wastewater, and related infrastructure on First Nation lands,” first reading December 11, 2023, <https://www.parl.ca/legisinfo/en/bill/44-1/c-61>; APTN News, “Alberta, Ontario don’t want feds to reintroduce First Nations safe drinking water law,” July 3, 2025, <https://www.aptnnews.ca/national-news/drinking-water-ontario-alberta-c-61/>.
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Chapter III

Structural Eugenics in Contemporary Alberta

Six Mechanisms in the Contemporary Architecture

Chapter II established the analytic framework: eugenics is broader than surgical sterilization; “structural eugenics” is the documented disability-studies term for policies producing eugenic effects without requiring eugenic intent; and consultation under CRPD Article 4(3) is a binding international standard that Alberta has not met in the development of contemporary disability policy. This chapter applies that framework to six specific Alberta policies currently in force or scheduled to come into force in 2026. The inventory is not exhaustive. The six mechanisms documented here are those for which the primary-source record is the most complete and the structural-eugenics mapping is the most direct.

Each section identifies the policy, the documented effect, and the historical eugenic mechanism it parallels. The analytic claim is the effect claim defined in Chapter II: that these policies produce measurable reductions in disabled people’s capacity to form households, to reproduce, to participate in community, to maintain long-term partnerships, and to access the material conditions of a livable life. The intent claim is not the claim made here. The effect claim is what the documented record supports.

The Cohabitation Rule

Under current AISH policy, the income of any adult sharing a household with an AISH recipient may be assessed as part of the household income calculation and used to reduce the recipient’s benefit. The rule applies regardless of the nature of the relationship: a married spouse, a common-law partner, a roommate sharing rent, an adult child temporarily living at home, an elderly parent in a granny suite, and a co-parenting partner who is not a romantic partner are all treated under the same household-income provision. The rule contains narrow exemptions, notably for paid live-in caregivers under documented arrangements; the exemptions do not cover practical roommate, co-parenting, or multi-generational household arrangements common in low-income, Indigenous, and immigrant communities.¹

On May 12, 2026, the Government of Alberta filed AR 96/2026, a new General Regulation governing both AISH and ADAP, in force on July 2, 2026. Section 8(4) of the regulation introduces what the government’s fact sheet describes as an “allowance for two-client couples living together”: where two adults in the same household each receive disability income assistance, each partner receives 88 percent of the maximum individual benefit. The reduction applies regardless of which program each partner is on.²

The mathematics, drawn from the government's own base rates, are as follows. Two AISH recipients sharing a household lose \$5,587.20 per year against the baseline of two unreduced AISH benefits. A mixed AISH-and-ADAP household loses \$7,699.20 per year. Two ADAP recipients lose \$9,811.20 per year. These figures are calculated before any employment income is considered and before the federal Canada Disability Benefit clawback discussed in section 3 of this chapter, which compounds against the same household.³

The structural effect is documented and direct. Partnership between disabled adults receiving AISH or ADAP is, after July 2, 2026, a \$5,587 to \$9,811 annual penalty against the household. Practical household-sharing arrangements between an AISH recipient and any other adult — romantic, familial, platonic — place the recipient's benefit at risk of reduction through the broader cohabitation rule. The recipient is, in the language of Inclusion Alberta's ADAP fact sheet, "structurally incentivized to remain alone."⁴

The historical eugenic mechanism the rule parallels is direct. The marriage prohibitions, immigration exclusions, cohabitation restrictions in residential institutions, and active provincial discouragement of family formation by disabled people documented in Chapter I operated through legal prohibition. The contemporary mechanism operates through financial penalty. The legal prohibition has been repealed; the financial mechanism that produces equivalent isolation has not been.

Sub-Survival Income: AISH, ADAP, and the Housing Crisis

AISH provides a maximum living allowance of \$1,940 per month in 2026. ADAP, the program to which all AISH recipients without severe and profound developmental disabilities, PDD eligibility, or palliative/terminal conditions will be transitioned on July 1, 2026, provides a maximum of \$1,740 per month. Recipients who are transitioned receive a transition payment maintaining the AISH rate until December 31, 2027; on January 1, 2028, the payment drops to the ADAP rate.⁵

Average one-bedroom rent in Edmonton and Calgary ranged from \$1,500 to \$1,900 in 2024, with documented year-over-year increases of 17.5 percent province-wide and up to 21.3 percent in three-bedroom markets, against a provincial average of \$2,138 per month for three-bedroom units. Alberta does not impose a residential rent-increase cap.⁶

The arithmetic produces the documented condition. A single AISH recipient on the maximum living allowance at the lower end of one-bedroom rent has, after rent, between forty and four hundred forty dollars per month for all other costs — food, utilities, transportation, medical out-of-pocket expenses, clothing, household supplies. Statistics Canada and Canada Food Price Report estimates for single-adult grocery costs in 2026 are eight hundred to one thousand dollars per month. Alberta utility estimates are three hundred eighty-five to five hundred ten dollars per month. The single recipient on private-market rent is operating, by these calculations, at a structural deficit of seven hundred fifty dollars or more per month.⁷

In Alberta's 2023–2024 fiscal year, 83.6 percent of AISH recipients had no employment income. The Government of Alberta's framing of ADAP as an empowerment program enabling Albertans with

disabilities to pursue employment operates against a documented baseline in which the great majority of recipients are not, on the available evidence, in a position to do so.⁸

The historical eugenic mechanism the income structure parallels is institutional segregation. The historical residential institutions documented in Chapter I — the Provincial Training School at Red Deer and the network of mental hospitals and guidance clinics — excluded disabled people from full community participation by removing them from the community. The contemporary income structure excludes disabled people from full community participation by setting income at a level that does not permit the housing, food, transportation, or social engagement that participation requires. The mechanism has changed from institutional confinement to economic confinement. The effect — exclusion from the conditions of community life — is the same.

The Canada Disability Benefit Clawback

The federal *Canada Disability Benefit Act* was passed in 2023 as a poverty-reduction measure for working-age Canadians with disabilities. The benefit pays up to \$200 per month, subject to a Disability Tax Credit prerequisite and an income-tested eligibility framework. The federal government, in introducing the benefit, explicitly requested that provinces and territories not reduce their own disability income payments when recipients also receive the federal benefit.⁹

In a letter to AISH recipients dated July 2, 2025, the Government of Alberta notified recipients that the Canada Disability Benefit would be treated as non-exempt income under AISH and ADAP. Recipients who did not demonstrate that they had applied for the CDB would see an equivalent \$200 deduction from their provincial benefits. Alberta is the only province or territory in Canada to claw back the Canada Disability Benefit dollar for dollar. Every other province and territory fully exempts the federal payment.¹⁰

The fiscal effect is direct. Applied to approximately 79,290 AISH recipients at \$200 per month, the clawback captures approximately \$190 million per year in federal transfer payments that would, in any other Canadian province, be received by disabled recipients. In Alberta, the federal investment is redirected to the provincial budget; the net benefit to the disabled recipient is zero.¹¹

The structural effect is to nullify federal poverty-reduction policy at the provincial level for the recipient population the federal policy was designed to serve. The historical eugenic mechanism the clawback parallels is the active provincial discouragement of disabled people from accessing resources available to non-disabled people. The historical version operated through institutional segregation. The contemporary version operates through the structural capture of federal transfer payments designed to lift disabled Albertans above the poverty line.

The FSCD Three-Year Wait

The Family Support for Children with Disabilities program funds respite, in-home support, therapy services, and equipment for Alberta children under eighteen with severe disabilities. It is the primary provincial mechanism through which families of disabled children access intervention services during the developmental years.¹²

In October and November 2024, Inclusion Alberta surveyed 540 families from 79 of Alberta's 87 electoral districts about their FSCD experience. The findings, published in January 2025, document an average wait of three years from application to full range of FSCD services. Eighty-five percent of families who had received the partial Family Support Services agreement that follows initial eligibility were still without a caseworker for the full Child Focused Services component. Fifty-eight percent of families reported negative effects from going without supports, including job loss, missed developmental windows, and measurable physical and mental health deterioration.¹³

The Auditor General of Alberta examined FSCD in 2022 and reported that caseworker training and oversight processes were inadequate and that families “may not receive the supports and services they qualify for.” The Government of Alberta received those findings in 2022. Caseworker caseloads have since been documented at four hundred families per worker against a standard reference of one hundred. Seven temporary caseworker contracts were not renewed in September 2024. The FSCD budget increase in Budget 2025 was 3.61 percent against a documented population-plus-inflation requirement of 7.3 percent — a real-terms reduction of approximately 3.69 percentage points.¹⁴

The structural effect is documented in the developmental literature on early intervention. The years between roughly two and five years of age constitute the period in which intervention services for developmental disability produce the largest measurable gains in functional outcomes. A three-year wait from application moves children past this window before services begin. A child diagnosed at four whose family applies the same day will not, on the documented average, receive full FSCD services until age seven — outside the window the program's own evidence base identifies as most consequential.¹⁵

The historical eugenic mechanism the FSCD wait parallels is the institutional intervention in disabled children's developmental trajectory documented in Chapter I. The historical version operated through institutionalization at the Provincial Training School and similar facilities, where children were separated from family and community on the asserted ground that institutional care was protective. The contemporary version operates through administrative wait time. The mechanism has changed from confinement to delay. The effect — the developmental trajectory the disabled child would otherwise have had is not the trajectory they receive — is the same.

PDD: Guardianship, Trusteeship, and the Unreported Waitlist

The Persons with Developmental Disabilities program supports adults eighteen and over with developmental disabilities of onset before age eighteen. It funds residential support, employment support, community access, and individualized care planning. PDD currently supports more than thirteen thousand adults in Alberta. The waitlist — the number of adults who are eligible for PDD but not receiving services — has not been publicly reported by the Government of Alberta in three years. The last published waitlist data are from 2021.¹⁶

Inclusion Alberta surveyed 540 PDD-connected individuals in late 2024 specifically because the Government would not release waitlist data. The findings, published in January 2025, document that seventy percent of respondents still need to meet with a caseworker to develop the outcome plan that determines their services. Forty-one percent of those who do receive supports report that the supports are

not helping. The PDD budget increase in Budget 2025 was 5.94 percent against the 7.3 percent population-plus-inflation requirement — a real-terms reduction of approximately 1.36 percentage points.¹⁷

The structural framework PDD operates within is the *Alberta Adult Guardianship and Trusteeship Act*. The Act establishes a substitute-decision-making framework for adults whose capacity to make personal and financial decisions is operationally impaired. The Act's framework conflicts with Article 12 (equal recognition before the law) of the CRPD, which the UN Committee on the Rights of Persons with Disabilities has identified, in General Comment No. 1, as requiring transition from substitute decision-making to supported decision-making across signatory jurisdictions.¹⁸

The historical eugenic mechanism the PDD framework parallels is the assertion of institutional authority over the lives of disabled adults. The Alberta Eugenics Board documented in Chapter I asserted authority to determine that disabled adults' reproductive lives were an appropriate site for state decision-making. The contemporary AGTA framework asserts authority over disabled adults' personal-and-financial decisions through a substitute-decision-making model that international human rights law has identified as requiring reform. The mechanism has changed from sterilization authority to substitute-decision-making authority. The operating premise — that disabled adults are appropriate subjects of external decision-making in ways non-disabled adults are not — is the operating premise of both periods.

Institutional Care: The Separation of Couples

The Alberta continuing care system, encompassing long-term care facilities (Continuing Care Home Type A) and designated supportive living facilities (Continuing Care Home Type B), operates with documented bed-availability constraints, facility-specialization patterns that distribute different levels of care across distinct facilities, and a fee-determination architecture based on couple's combined-income calculations. The combination produces a documented pattern in which spouses are forced to live apart when one requires institutional care that the other does not.¹⁹

In June 2019, CBC News reported the case of Martha and Willard Farnell of Calgary, Alberta. The Farnells had been married for sixty-six years. Following Willard's dementia diagnosis and admission to a long-term care facility approximately twenty minutes' drive from Martha's residence, the Alberta continuing care system did not provide a mechanism through which the couple could continue to share accommodation. The CBC report documented Sweden's policy position treating forced spousal separation in long-term care as inhumane, and the Netherlands "Living Apart Together" suites model that permits couples to live in separate but connected rooms or apartments within the same care facility.²⁰

The peer-reviewed medical literature documents that severe loneliness affects up to forty-two percent of long-term-care residents, against ten percent prevalence in older adults living in the community. The mortality and morbidity consequences of social isolation in this population are documented at scale.²¹

Ontario's long-term care regulatory framework, since January 1, 2018, has required every long-term care home in the province to have at least two beds available for spouses. Alberta has no equivalent requirement. The provincial Government's own acknowledged capacity need is ten thousand new continuing-care spaces over three years; documented commitments are at a small fraction of that figure.²²

The historical eugenic mechanism the institutional-care separation pattern parallels is direct. The cohabitation restrictions enforced in the historical residential institutions documented in Chapter I — the Provincial Training School, the network of mental hospitals — separated disabled adults from spouses, family, and community as a matter of institutional policy. The contemporary version separates senior spouses, one or both of whom is disabled, through bed-allocation and facility-specialization mechanisms that the policy architecture characterizes as administrative necessity. The mechanism has changed from policy prohibition to administrative allocation. The effect — partnered disabled adults are separated from their partners by the institutional system at the moment they most need continued partnership — is the same.

What This Chapter Establishes

The six mechanisms inventoried in this chapter are not the complete inventory of structural eugenics in contemporary Alberta. They are the inventory for which the primary-source record is the most complete. Each is documented in current Alberta legislation, regulation, or program policy. Each is verified against current government data or against survey work conducted by Inclusion Alberta, the Auditor General of Alberta, or the peer-reviewed medical literature. None of the six requires speculative inference; each operates as documented.

Three observations follow.

First, the six mechanisms operate together. The cohabitation rule restricts partnership. Sub-survival income restricts community participation. The CDB clawback nullifies federal poverty-reduction intervention. The FSCD wait restricts developmental trajectory. PDD guardianship restricts adult autonomy. Institutional care separation restricts continued partnership in aging. Taken together, they constitute an architecture in which disabled people are structurally separated from the relational, developmental, financial, and social conditions of full life across the lifecycle. The architecture is not the product of a single decision. It is the cumulative product of decisions taken over decades, each defensible in isolation, none requiring eugenic intent.

Second, the historical mechanisms documented in Chapter I have direct parallels in this inventory. The marriage prohibitions of the 1928–1972 period are paralleled by the cohabitation rule. The institutional segregation of the Provincial Training School period is paralleled by the income and housing structure that prevents community-based independent living. The institutional cohabitation restrictions are paralleled by the long-term-care separation pattern. The institutional authority over disabled adults’ lives asserted by the Eugenics Board is paralleled by the Adult Guardianship and Trusteeship Act substitute-decision-making framework. The developmental intervention of the historical institutionalization period is paralleled by the FSCD wait. The active discouragement of disabled family formation documented in the 1928–1972 record is paralleled by the integrated effect of all six contemporary mechanisms.

Third, none of the six mechanisms documented here was developed through a consultation process meeting the standard CRPD Article 4(3) requires. The cohabitation rule was filed by regulation on May 12, 2026, ten days after the Inclusion Alberta media release identifying Bill 12 as eroding disability rights, and was not preceded by consultation with disability community representative organizations on its

specific 88-percent mechanism. The CDB clawback was communicated to recipients by letter on July 2, 2025, without consultation. The Bill 12 framework producing the AISH-to-ADAP transition was passed on December 11, 2025 over the documented opposition of every major Alberta disability advocacy organization. The FSCD, PDD, and institutional-care frameworks operate within budget and policy cycles that do not, on the available record, include the consultation through representative organizations the Convention requires.

The mechanism has changed. The architecture has not.

Notes — Chapter III

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5. Government of Alberta, AISH and ADAP program documentation, 2025–2026; The Alberta Disability System Breakdown, *Income Crossover Analysis*, revised May 2026, on the transition payment expiring December 31, 2027.
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9. *Canada Disability Benefit Act*, SC 2023, c 9; Employment and Social Development Canada, Canada Disability Benefit program documentation, 2025.
10. Government of Alberta letter to AISH recipients, July 2, 2025 (notification of CDB treatment as non-exempt income); CBC News, “Federal disability benefit to be deducted from AISH payments,” Bellefontaine, M., February 2025; Alberta Doctors’ Digest, “Pushing back against changes that make life harder for disabled Albertans,” November/December 2025; Gillian Petit, “The Canada Disability Benefit and Provincial Clawback: A Fiscal Transfer Analysis,” *Canadian Tax Observatory*, April 22, 2026.
11. Calculations based on Government of Alberta AISH Caseload Open Data (approximately 79,290 recipients as of September 2025) and the \$200/month maximum CDB payment.
12. Government of Alberta, Family Support for Children with Disabilities program documentation, alberta.ca.
13. Inclusion Alberta, *Experiences with Family Support for Children with Disabilities* (FSCD Survey Report), January 2025. 540 respondents from 79 of 87 Alberta electoral districts.
14. Auditor General of Alberta, *Report on FSCD*, 2022 (caseworker training and oversight findings); Hold My Hand Alberta / The Albertan News, “Some Alberta caseworkers that help disabled children have 100s of clients,” September 16, 2024; Government of Alberta, Budget 2025; Inclusion Alberta, Budget 2025 analysis.
15. The developmental early-intervention window for children between approximately ages two and five is documented across the developmental disability literature. See, e.g., discussions in the FSCD Survey Report (Inclusion Alberta, January 2025) and the broader developmental pediatrics literature on intervention timing.

16. Inclusion Alberta / ALIGN, *PDD Current State: Critical and Urgent: Experiences with Persons with Developmental Disabilities*, January 2025; CBC News, “Provincial support delays are hurting disabled Albertans and families, reports find,” Bellefontaine, M., January 23, 2025.
17. Inclusion Alberta / ALIGN, *PDD Current State*; Government of Alberta, Budget 2025; Inclusion Alberta, Budget 2025 analysis.
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19. Government of Alberta, continuing-care framework, including *Continuing Care Act* and related regulations; The Alberta Disability System Breakdown, *Building Up: A Blueprint for the Alberta Accessibility and Inclusion Act*, Part One, Section 1.10.
20. Melissa Mancini, “Separating Couples in Long-Term Care: Some Nations Call It Inhumane, in Most of Canada It’s Routine,” CBC News, June 2, 2019, <https://www.cbc.ca/news/health/national-long-term-care-couples-housing-1.5159102>. The CBC report documents the Farnell case and comparator-jurisdiction approaches.
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Chapter IV

The AISH-to-ADAP Transition and the Consultation Standard

Bill 12, the Premier's Council, and the UN Committee Warning

The Transition

On December 9, 2025, the Alberta Legislative Assembly passed Bill 12, the *Financial Statutes Amendment Act, 2025 (No. 2)*. The bill received Royal Assent on December 11, 2025 and was enacted as SA 2025 c20.¹

Bill 12 effects what Inclusion Alberta has described as the most consequential restructuring of disability income in Alberta in a generation. Effective July 1, 2026, all current AISH recipients will be transitioned to the Alberta Disability Assistance Program (ADAP). The transition affects approximately 79,290 Albertans — the population the Government of Alberta has formally assessed as having severe and permanent disabilities preventing employment. Section 12.8 of Bill 12 removes the right of appeal for the transition decision. Recipients who wish to remain on AISH must reapply through a Medical Review Panel whose decisions are, on the legislation's own terms, final and not subject to appeal to an independent body.²

This chapter does not catalogue the substantive content of Bill 12 again; the structural mechanisms it activates — sub-survival income, cohabitation rules, employment-exemption reductions, federal-transfer capture — are documented in Chapter III. This chapter is concerned with a different question: whether the procedural framework under which Bill 12 was developed and passed met the consultation standard Canada has formally accepted under the United Nations Convention on the Rights of Persons with Disabilities. The conclusion the documented record supports is that it did not.

The Government's Consultation Claim

The Government of Alberta has, throughout the period of Bill 12's development and passage, advanced a single foundational rhetorical claim about ADAP. The claim appears on the program's public information page, in the May 12, 2026 Fact Sheet, and in ministerial correspondence to AISH recipients and to municipal councils. It reads:

ADAP is being thoughtfully designed, based on input from Albertans with disabilities...
ADAP public engagement concluded September 12, 2025. Thousands of Albertans participated in engagement via roundtables, telephone town halls, written submissions and

the online survey. Public engagement was undertaken in addition to engagement with disability advocates and service providers, including through roundtables and meetings.³

The structure of the rhetorical claim has three components. First, the program reflects what Albertans with disabilities asked for. Second, the engagement was numerically substantial — “thousands of Albertans.” Third, the engagement was augmented by direct meetings with advocates and service providers. Every government defence of Bill 12 against criticism — including responses to municipal motions, to media inquiry, and to written correspondence from constituents — rests on this foundation.⁴

What the Consultation Standard Requires

Chapter II of this piece establishes the binding international framework. CRPD Article 4(3), to which Canada has been bound since ratification on March 11, 2010, requires State Parties to “closely consult with and actively involve persons with disabilities, including children with disabilities, through their representative organizations” in the development and implementation of legislation and policies concerning persons with disabilities. The CRPD Committee’s General Comment No. 7 (2018) elaborates the standard. Consultation must be substantive and ongoing rather than notification-based. It must occur in the design phase, not only the comment phase. It must be conducted through representative organizations the disability community itself recognizes — not through individuals selected by the consulting authority. And it must, as a matter of substantive engagement, produce outcomes that the affected population can reasonably recognize as reflecting their input.⁵

Alberta’s domestic framework for meeting this standard has a specific institutional embodiment. The *Premier’s Council on the Status of Persons with Disabilities Act*, RSA 2000, c P-21, establishes the Premier’s Council as the standing statutory advisory body for disability policy. The Council is required by statute to advise on the development of disability legislation and programs. Its structure includes specific subcommittees for particular populations, including Indigenous people with disabilities. Its consultation processes, even when conducted by online survey, are framed as advisory input feeding into a multi-stakeholder analytic process led by disability community members themselves.⁶

The question this chapter examines is whether the consultation process undertaken by the Government of Alberta in connection with Bill 12 met the standard the Convention requires and the standard the province’s own statutory framework contemplates. The documented record establishes that it did not.

The Documented Gaps Between the Claim and the Standard

The Premier’s Council bypass. The Government of Alberta did not, in its public-facing consultation materials or in its responses to municipal advocacy, route the ADAP consultation through the Premier’s Council. The Council’s 2023–2026 Strategic Plan survey, conducted through its standing statutory framework, received 381 responses. The ADAP consultation, conducted through a parallel bespoke process the government directly controlled, is reported to have drawn “thousands.” The disparity reflects where the government directed participation effort: away from the standing mechanism, toward the process the government itself designed. The Premier’s Council exists. Its statute requires it to advise on the development of disability policy. The government chose to route ADAP consultation outside the Council. The choice itself is a documentation gap; a government that relied on its standing statutory body for this consultation would have a defensible structural answer to many of the further concerns documented below.⁷

The undisaggregated “thousands.” The government’s claim that “thousands of Albertans” participated in the consultation is not, on the public record, broken down by recipient status. The campaign has formally requested the participant breakdown: current AISH recipients, current recipients of other disability benefits, family members and caregivers of recipients, service providers and advocates, non-recipient Albertans with disabilities, and other. The government has not provided this breakdown. The Convention’s standard requires consultation with persons with disabilities through their representative organizations; the consultation cannot be verified as meeting that standard without data on who, specifically, participated.⁸

Provisions introduced after consultation closed. The consultation window closed on September 12, 2025 (a date the engagement page subsequently and quietly changed to September 14, 2025). Bill 12 was introduced on November 25, 2025, more than two months after the consultation closed. Key operative provisions of Bill 12 — including the removal of independent appeal rights through the abolition of the Citizens Appeal Panel, and the substantial revision of employment income exemption figures from the levels indicated in the August 2025 Discussion Guide — were not in the consultation materials on which participants based their input. The CRPD Article 4(3) standard requires consultation on the substantive content of legislation. Consultation on a discussion guide whose provisions are subsequently revised does not, on its face, meet that standard.⁹

The petition tabled at the Legislature. In October 2025, Calgary-based advocate Darryl Learie initiated a petition against the ADAP framework. The petition was circulated through disability community networks across Alberta and was tabled at the Alberta Legislature with documented signatures. The petition constitutes a documented empirical statement, by named Albertans with disabilities and their families, against the program the government’s consultation claim says reflects their input. A consultation process producing a program against which the population it consulted petitions does not, on its face, reflect what that population asked for.¹⁰

The opposition record across the disability sector. Inclusion Alberta’s November 26, 2025 media release stated that “Bill 12 erodes rights and deepens poverty for Albertans with disabilities.” The Alberta Medical Association, the Alberta Union of Provincial Employees, Disability Action Hall, the People’s

Alliance for Disabled Albertans, MS Canada, and Friends of Medicare Lethbridge have publicly opposed Bill 12. The Chair of Friends of Medicare Lethbridge, Bev Muendel-Atherstone, has characterized the transition as “the equivalent of being found guilty of fraud or malingering without having a trial or sentencing.” No major Alberta disability advocacy organization has publicly endorsed the Bill 12 framework. A municipal coalition including Edmonton, Calgary, Lethbridge, Camrose, Claresholm, Red Deer, and Medicine Hat has called for the AISH-to-ADAP transition to be paused pending meaningful consultation.¹¹

The engagement page record. The Alberta government’s ADAP engagement page — the public-facing documentation of the consultation the government’s rhetorical claim rests on — was rewritten from an active consultation portal to a look-back archive within seven days of the consultation closing. The page’s stated survey end date was quietly changed from September 12 (the date in the Discussion Guide and on the page during the consultation) to September 14, 2025. The page sat unchanged for 206 days through the introduction of Bill 12, the passage of Bill 12, and the March 2026 budget announcement — none of which were added to the page that documents the consultation feeding into those milestones. Since April 12, 2026, the page has been blocked from public archiving via the Wayback Machine. The pattern is documented in the campaign’s *Engagement Page Record: A Forensic Timeline*.¹²

The UN Committee Warning, March 26, 2025

On March 26, 2025, nine months before Bill 12’s passage in the Alberta Legislative Assembly, the United Nations Committee on the Rights of Persons with Disabilities issued its Concluding Observations on the combined second and third periodic reports of Canada. The Committee, in its capacity as the treaty body monitoring CRPD implementation, examined the operation of Canadian disability law and policy across federal, provincial, and territorial jurisdictions.¹³

Among the Committee’s specific recommendations, one is directly relevant to the structural circumstance under which Bill 12 was developed and passed. The Committee recommended that Canada “establish legally defined mechanisms ensuring compliance with the CRPD by all provinces and territories, and condition fiscal transfers from the Federal level to provinces and territories on their adherence to the CRPD.”¹⁴

The Committee’s recommendation addresses, in operational terms, the constitutional reality that human rights treaties ratified by the federal government are not automatically binding on provincial governments in their areas of constitutional jurisdiction. The federal government ratified the Convention; provincial governments operate the substantial portion of disability policy directly affecting recipients. Where a province develops legislation in an area engaging the rights protected under the Convention without meeting the Convention’s consultation standard, federal mechanisms to ensure compliance become the mechanism by which Canada’s treaty obligations are operationalized at the provincial level.

Canada has not, on the documented public record, acted on the Committee’s recommendation. No federal mechanism exists to condition fiscal transfers to provinces on CRPD compliance. The province that bypassed its own standing statutory advisory body in the development of the most consequential disability legislation in a generation faced no federal-level operational consequence for doing so. The Committee’s

March 26, 2025 warning of exactly this structural risk preceded Bill 12's passage by nine months. Bill 12 proceeded.

The Removal of Appeal Rights

Bill 12's most operationally consequential provision, after the transition itself, is its elimination of independent appeal rights for the affected population.

Under the AISH framework that existed before Bill 12, recipients who disagreed with an eligibility decision had recourse to the Citizens Appeal Panel — a quasi-judicial body composed of community members with relevant expertise, operating at arm's length from the AISH program and from the Government of Alberta. The Panel's decisions were binding on the program. The Alberta Ombudsman's public-facing description confirmed the appeal route: "Most decisions about your benefits can be appealed through the Appeals Secretariat."¹⁵

Bill 12 abolished the Citizens Appeal Panel as the appeal body for AISH eligibility decisions. In its place, the legislation establishes a Medical Review Panel composed of medical professionals appointed by the Government. The Medical Review Panel reviews disputed eligibility decisions. Its decisions are final. There is no independent appeal from a Medical Review Panel decision; judicial review remains available only on procedural grounds, not on the merits.¹⁶

Section 12.8 of Bill 12 specifically provides that the transition decision — the determination that an AISH recipient is reassigned to ADAP rather than retained on AISH — cannot be appealed. The CBC News reporting on August 21, 2025 quoted the ADAP Discussion Guide describing the Medical Review Panel as making decisions that "are final and not subject to appeal."¹⁷

Bill 12 also gives the Government of Alberta authority to limit which other program decisions can be appealed under the post-transition framework. The appeal-rights structure that operated as the procedural protection for AISH recipients across decades no longer exists in the form it existed in as recently as 2024.¹⁸

The CRPD Article 13 protection of access to justice for persons with disabilities is implicated directly by this provision. So is the structural conflict of interest inherent in the new arrangement: the party with the financial interest in reducing the AISH caseload (the Government of Alberta) controls the panel that determines who stays on AISH (the Medical Review Panel), and the Medical Review Panel's decisions are final. The independent check that the Citizens Appeal Panel provided no longer exists.

The Parallel to 1998

Chapter I of this piece documented a previous legislative moment in Alberta in which the provincial government, faced with the fiscal implications of a court decision recognizing rights of persons with disabilities, responded by attempting to constrain those rights through legislation. In 1998, the Klein government's response to the wave of sterilization lawsuits opened by the *Muir* judgment was Bill 26 of that year, the *Institutional Confinement and Sexual Sterilization Compensation Act*, which attempted to invoke section 33 of the Charter to cap damages and impose a 180-day limitation period. The 1998 Bill 26 was withdrawn within twenty-four hours under public pressure. The underlying impulse — that the appropriate institutional response to a population's assertion of rights is the constraint of those rights — was, in that moment, defeated in public.¹⁹

Bill 12 in 2025 operates on the same impulse through different mechanisms. The 1998 bill would have invoked section 33 of the Charter explicitly. The 2025 framework operates without the visible constitutional override the 1998 bill required, but the structural effect is comparable: the rights protected under the CRPD — specifically Articles 4(3), 12, 13, 19, 21, and 28 — are not honoured in the operation of the legislation. The recipient population was not consulted through representative organizations in the development phase. The legal-capacity protections of Article 12 are not honoured by the reversal of 79,290 prior findings of permanent disability without individual reassessment. The access-to-justice protections of Article 13 are not honoured by the elimination of independent appeal. The community-living protections of Article 19 are not honoured by the income reduction to ADAP levels. The accessible-information protections of Article 21 are not honoured by the absence of plain-language documentation of program operational rules. The adequate-standard-of-living protections of Article 28 are not honoured by the transition to sub-survival benefit levels.²⁰

In 1998, the constraint of rights was attempted explicitly and failed. In 2025, the constraint of rights was effected administratively and succeeded. The 1998 effort had to be defended at the constitutional level. The 2025 effort proceeded without that requirement, because the operational mechanisms were located in the financial structure of the program rather than in an explicit Charter override. The structural outcome is the same. The procedural visibility is different.

What This Chapter Establishes

Four observations follow from this chapter.

First, the consultation undertaken by the Government of Alberta in connection with Bill 12 did not meet the standard CRPD Article 4(3) requires. The standing statutory advisory body was bypassed. The participant breakdown supporting the “thousands of Albertans” claim has not been published. Key program provisions were introduced after consultation closed. A petition against the program was tabled at the Legislature. No major Alberta disability advocacy organization endorsed the framework. The engagement page itself has been altered in ways that are visible in the historical archive and that the government has not publicly explained.

Second, the international treaty body whose mandate is to monitor implementation of the Convention had specifically warned Canada about exactly this structural risk nine months before Bill 12 passed. The Committee's recommendation of legally defined federal mechanisms to condition fiscal transfers on provincial CRPD compliance was issued on March 26, 2025. Bill 12 passed on December 9, 2025. The Committee's warning was on the record. The provincial framework proceeded.

Third, the elimination of independent appeal under Bill 12 transfers final adjudicatory authority over AISH eligibility to a panel appointed by the same Government of Alberta that benefits financially from reducing the AISH caseload. The structural conflict of interest is built into the legislation. The procedural protection that the Citizens Appeal Panel provided for decades has been removed. The access-to-justice protections the Convention requires under Article 13 are not, on the documented record, honoured by this arrangement.

Fourth, the parallel to 1998 is not metaphorical. The Klein government's 1998 Bill 26 attempted to constrain the rights of sterilization survivors through explicit Charter override; it was withdrawn under public pressure within twenty-four hours. The Smith government's 2025 Bill 12 constrains the rights of contemporary disabled Albertans through administrative mechanisms that operate below the threshold of explicit constitutional confrontation. The structural impulse — that the appropriate institutional response to a population's assertion of rights is the constraint of those rights — is the same. The visibility of the impulse is what has changed.

The chapter that follows examines the upstream condition without which the consultation standard cannot operate: the right of the affected population to know, in advance, what they are being asked to consent to.

Notes — Chapter IV

1. Legislative Assembly of Alberta, Bill 12, *Financial Statutes Amendment Act, 2025 (No. 2)*, 31st Legislature, Second Session; passed at Third Reading December 9, 2025; received Royal Assent December 11, 2025; enacted as SA 2025 c20. Bill text and legislative status available at the Legislative Assembly of Alberta document repository.
2. Bill 12, section 12.8 (removal of appeal of the AISH-to-ADAP transition); see also CBC News (Bellefontaine), August 21, 2025, reporting on the ADAP Discussion Guide; Inclusion Alberta, “Bill 12 erodes rights and deepens poverty for Albertans with disabilities,” media release, November 26, 2025; Zachary Weeks, “Bill 12 and ADAP: What Alberta’s New Disability Law Means for AISH Recipients,” November 28, 2025. The 79,290 figure is from Government of Alberta, AISH Caseload Open Data, September 2025.
3. Government of Alberta, Alberta Disability Assistance Program public information page, alberta.ca/alberta-disability-assistance-program; Government of Alberta, Assisted Living and Social Services, ADAP Fact Sheet, May 12, 2026. The same foundational consultation claim appears across both sources.
4. See, for example, Minister Jason Nixon’s correspondence to AISH recipients (including the April 3, 2026 letter to Genevieve Bissonette) and Government of Alberta responses to municipal council motions calling for the transition to be paused.
5. *Convention on the Rights of Persons with Disabilities*, Article 4(3); Canada signed March 30, 2007, ratified March 11, 2010; Committee on the Rights of Persons with Disabilities, General Comment No. 7 (2018) on the participation of persons with disabilities through their representative organizations, CRPD/C/GC/7. (Discussed at length in Chapter II.)
6. *Premier’s Council on the Status of Persons with Disabilities Act*, RSA 2000, c P-21; Government of Alberta, Premier’s Council on the Status of Persons with Disabilities, Annual Report 2022–2023 (documenting the subcommittee structure, including the Indigenous people with disabilities subcommittee, and the 2023–2026 Strategic Plan survey methodology and response total).
7. Government of Alberta, Premier’s Council Annual Report 2022–2023 (381 Strategic Plan survey responses); The Alberta Disability System Breakdown, *Consultation Gap Rebuttal*, April 2026, Layer 5 (the Premier’s Council bypass).
8. The Alberta Disability System Breakdown, *Consultation Gap Rebuttal*, April 2026, Layer 2 (the undisaggregated participant claim) and Layer 4 (the petition as documented empirical rebuttal).
9. Government of Alberta, *Transforming Disability Income Assistance in Alberta: Discussion Guide*, August 13, 2025; Government of Alberta, ADAP engagement page (consultation window August 13 – September 12, 2025); Inclusion Alberta, *ADAP Fact Sheet*, October and December 2025 (documenting the revisions to employment income exemption figures and the appeal-rights provisions that emerged after consultation closed). Bill 12 introduced November 25, 2025.
10. Darryl Learie, Petition Against the Provincial Government’s New Alberta Disability Assistance Program (ADAP), October 8, 2025, polioalberta.ca; tabled at the Alberta Legislature with documented signatures.
11. Inclusion Alberta, “Bill 12 erodes rights and deepens poverty for Albertans with disabilities,” media release, November 26, 2025; MS Canada, “MS Canada calls to protect disability supports in Alberta,” November 28, 2025; Alberta Doctors’ Digest, “Pushing back against changes that make life harder for disabled Albertans,” November/December 2025; Disability Action Hall, 2025 record at actionhall.ca/2025/; Bev Muendel-Atherstone (Friends of Medicare Lethbridge), Lethbridge Herald, October 2025. Municipal coalition documented in The Alberta Disability System Breakdown Coalition Ledger, May 2026.
12. The Alberta Disability System Breakdown, *The Engagement Page Record: A Forensic Timeline*, May 2026.
13. United Nations Committee on the Rights of Persons with Disabilities, Concluding Observations on the combined second and third periodic reports of Canada, March 26, 2025 (document CRPD/C/CAN/CO/2-3, citation to be

verified at OHCHR before final edition).

14. CRPD Committee Concluding Observations on Canada, March 26, 2025, recommendation on legally defined federal mechanisms and the conditioning of fiscal transfers on provincial CRPD adherence.
15. Alberta Ombudsman, Appeals Secretariat information page (public-facing description of pre-Bill-12 appeal route); The Alberta Disability System Breakdown, *AISH Evidentiary Report Series*, April 2026, section on the pre-Bill-12 Citizens Appeal Panel.
16. Bill 12, sections governing the Medical Review Panel and the abolition of the Citizens Appeal Panel; Inclusion Alberta, *ADAP Fact Sheet*, December 2025; Zachary Weeks, “Bill 12 and ADAP,” November 28, 2025.
17. Bill 12, section 12.8; CBC News (Bellefontaine), August 21, 2025 reporting on the ADAP Discussion Guide.
18. The Alberta Disability System Breakdown, *AISH Evidentiary Report Series*, April 2026, section comparing pre-Bill-12 and post-Bill-12 appeal rights structures.
19. See Chapter I and its associated endnotes for the full record of the 1998 Bill 26 episode (the *Institutional Confinement and Sexual Sterilization Compensation Act* of 1998), distinct in subject and provenance from the 2018 NDP Bill 26 (*An Act to Combat Poverty and Fight for Albertans with Disabilities*).
20. See *Convention on the Rights of Persons with Disabilities*, Articles 4(3), 12, 13, 19, 21, and 28; The Alberta Disability System Breakdown, *Charter and CRPD Analysis*, April 2026, mapping the articles to the documented operational practices under Bill 12.

Chapter V

The Information Asymmetry Condition

The Right to Know Before Consent

The Upstream Condition

Chapter IV closed on a forward reference. The chapter that preceded examined whether the consultation undertaken in connection with Bill 12 met the standard CRPD Article 4(3) requires. The documented record established that it did not. That chapter closed by naming the upstream condition without which the consultation standard cannot operate: the right of the affected population to know, in advance, what they are being asked to consent to.

This chapter examines that condition. The claim it advances is narrow and structural. Consent presupposes information. A population that is not in a position to know the operational rules of the program being designed for them, the criteria against which their continued eligibility will be assessed, or the standards by which the documentation they are required to produce will be evaluated, cannot meaningfully consent to the program. The consultation standard CRPD Article 4(3) requires is operationally hollow where the upstream information condition is not met. The hollow standard is the standard the Government of Alberta operated under in 2025. The chapter that follows documents how.

The Standards That Apply

Two articles of the Convention on the Rights of Persons with Disabilities are directly engaged. Article 9, the accessibility provision, requires State Parties to take appropriate measures to ensure that persons with disabilities have access, on an equal basis with others, to information and communications, including information and communications technologies and systems. Article 21, the freedom of expression and access to information provision, requires State Parties to ensure that information intended for the general public is provided in accessible formats and technologies appropriate to different kinds of disabilities, in a timely manner and without additional cost.¹

The domestic framework adds further specificity. The federal *Accessible Canada Act*, SC 2019, c 10, enacted as the domestic operationalization of Canada's CRPD obligations in the federal sphere, establishes the statutory goal of a Canada without barriers by January 1, 2040. The Act binds federally regulated organizations and federally delivered programs. The federal components implicated in the disability-support architecture — the Canada Revenue Agency, Service Canada, the Disability Tax Credit, and the Canada Disability Benefit — are within scope.²

At the provincial level, the *Alberta Human Rights Act* prohibits discrimination on the basis of physical and mental disability in the provision of government services. Section 4 establishes an affirmative duty of accommodation triggered when the service provider is on notice that a barrier exists — not when the affected individual files a formal request. At the constitutional level, Section 7 of the Canadian Charter of Rights and Freedoms protects against deprivation of life, liberty, or security on the basis of evidence the affected person cannot review, criteria they cannot evaluate, or processes in which they cannot meaningfully participate. The Supreme Court of Canada in *Eldridge v. British Columbia (Attorney General)*, [1997] 3 SCR 624, established that Section 15 of the Charter requires substantive equality — not merely identical text on paper, but the actual capacity of protected groups to exercise their rights on equal terms.³

Plain-language standards operationalize what accessibility means for written communications. The federal Treasury Board Secretariat recommends a Grade 8 reading level or below for general public communications. The disability-advocacy community recommends Grade 6 or below for communications directed at populations with cognitive or intellectual disabilities. The federal Plain Language Action and Information Network defines plain language as writing that the intended audience can read, understand, and act on the first time they read it. Statistics Canada’s Programme for the International Assessment of Adult Competencies (PIAAC) data places approximately forty-nine percent of Canadian adults below the literacy threshold required for everyday government and health communications.⁴

The Reading-Level Gap

Independent readability analysis of Government of Alberta AISH program documentation, ADAP transition information, and federal Canada Disability Benefit application materials places the typical reading level between Grade 12 and Grade 16 — that is, university-undergraduate to graduate-level English. A document at this reading level is comprehensible to approximately twelve percent of Canadian adults at first-read full comprehension.⁵

The reading level matters because of who is in the AISH caseload. The Government of Alberta’s own caseload data as of September 2025 identifies primary medical conditions across the 79,290 recipients as follows: 22,419 (28.3 percent) with cognitive disorders as their primary diagnosis, including autism, intellectual disability, acquired brain injury, and severe dementia; 23,940 (30.2 percent) with mental illness as their primary diagnosis, including major depressive disorder, severe anxiety, schizophrenia, and post-traumatic stress disorder; approximately 24,800 (31.3 percent) with physical disability, including mobility, sensory, and chronic-illness conditions; and approximately 8,131 (10.2 percent) with other or multiple primary diagnoses. Combined, more than 58 percent of the AISH caseload — 46,359 people — have a primary diagnosis of cognitive disorder or mental illness.⁶

The defining clinical features of these conditions include difficulty with sustained executive function, difficulty processing complex written communication, difficulty initiating and completing multi-step administrative tasks, difficulty navigating institutional systems without external support, and difficulty maintaining sustained attention across multi-page documents. These are not aspersions. They are the diagnostic criteria. They are the basis on which the Government of Alberta approved each recipient as

eligible for AISH in the first place.

The same Government of Alberta then designed a compliance system requiring those same recipients to read forms written at Grade 12 to 16 reading level, to complete multi-step paperwork chains across multiple agencies, to find a family doctor in a province where approximately 650,000 people have no family physician, to pay physician fees out of pocket on an income below the poverty line, and to navigate phone trees, online portals, and appeal processes without integrated accommodation. The defining clinical features of the population the program was designed to serve are the defining features the program then requires the population to override in order to remain on it.⁷

What Was Not Published Before the Transition

The information that the Government of Alberta did not publish, in any form that a prospective ADAP recipient could review before the consultation closed in September 2025, is the category of information most directly relevant to the consent question Chapter IV examined. The list is documented in the campaign's *AISH Evidentiary Report Series* and the *Compound Failure Brief*; the items below are reproduced with the campaign's primary-source verification.⁸

The basis for the reversal of prior findings. Every AISH recipient was assessed and approved by the Government of Alberta's formal adjudication process. The ADAP transition reverses those findings for all 79,290 recipients simultaneously. The Government has not published, in any form, the medical, administrative, or legal basis on which the prior findings are treated as insufficient to maintain eligibility without reapplication. The reversal is operative; the justification is undocumented.

The criteria distinguishing AISH from ADAP eligibility. Minister Jason Nixon confirmed in correspondence dated April 3, 2026 that the employment income exemption calculations under the post-transition framework would be set in a Ministerial Order "later this spring." The transition is effective July 1, 2026. The operational rules governing the income reduction structure that will apply to 79,290 people were not published when those people were consulted in the engagement window that closed in September 2025. They were not published when Bill 12 was passed in December 2025. They were not published as the campaign's most recent documentation of the matter went to publication in May 2026.⁹

The Medical Review Panel adjudication standards. The Panel that replaces the Citizens Appeal Panel as the final decision-maker on AISH eligibility under the post-transition framework will apply criteria the Government has not published. The specific criteria the medical adjudicator applies to determine "substantial limitation of ability to earn a livelihood" have not been disclosed. The evidentiary thresholds for "permanence" of a condition have not been disclosed. The assessment standards for fluctuating conditions — which do not present consistently across any single assessment period and which characterize many of the mental health and chronic-pain conditions in the caseload — have not been disclosed. The criteria for determining "likely improvement," a determination that requires the adjudicator to make a prognosis the applicant's own physicians may not have made, have not been disclosed.¹⁰

The structural “framing-amenable” criterion. The campaign’s *Compound Failure Brief* identifies an operative criterion that the Medical Review Panel will, in functional effect, apply: not whether a recipient’s disability is real or severe (the existing file already establishes that, and the Government has accepted the equivalent established record for every recipient in an exempted category) but whether a recipient can be plausibly framed as a candidate for employment. The exemption categories themselves identify the criterion: recipients over sixty are exempted because their employment trajectory is no longer considered relevant; PDD recipients are exempted because their developmental disability is considered conclusive of unemployability; continuing-care residents are exempted because their living situation is conclusive; palliative recipients are exempted because their prognosis is conclusive. The transitioning cohort is the cohort the Government considers framing-amenable. The criterion is not in any published document. Its operation is documented in the structure of the exemptions.¹¹

The recipient’s position. The applicant whose file is being newly evaluated under this unpublished criterion has no opportunity to review what is being applied to them, no opportunity to respond to it, and no independent body to which an erroneous application can be appealed. The criterion is being applied by appointees of the ministry whose budget benefits from each ADAP placement, in the absence of the appeal panel that historically reviewed decisions of this kind. The Government’s existing record on the recipient — the record establishing that the same Government previously found this person unable to sustain a livelihood — is held in the same ministry’s filing cabinet as the new determination is being made.

The Compounding Effect Across Programs

The information-asymmetry pattern is not unique to AISH and ADAP. It compounds across the federal-provincial disability-support chain. The federal Canada Disability Benefit requires prior approval for the Disability Tax Credit. The DTC requires a physician to complete the T2201 form. Physician fees for T2201 completion are unregulated and range from approximately fifty to three hundred dollars or more across the province. The Government of Canada’s 2024 Disability Advisory Committee report documents a twenty-four percent completion rate for online DTC applications — equivalently, a seventy-six percent abandonment rate. The Disability Advisory Committee identifies administrative complexity as a documented access barrier rather than a measure of ineligibility.¹²

The same population that must complete the DTC to qualify for the CDB is the population whose CDB Alberta then claws back dollar-for-dollar from the provincial AISH or ADAP cheque. The documentation chain is documented in the campaign’s *Access Barrier Report* and traced through timestamped correspondence in a primary-source case file spanning 2019 to 2026. The chain’s practical effect is that a recipient who succeeds in completing all of the federal access requirements receives zero net financial benefit, and a recipient who fails in completing them sees their AISH cheque reduced by two hundred dollars per month regardless.¹³

The same pattern operates in the child and developmental-disability programs. The Family Support for Children with Disabilities (FSCD) and Persons with Developmental Disabilities (PDD) program forms inherit the same readability and accessibility issues with one structural addition: the parent or guardian

completing the form is often themselves managing the disability of the child or family member requiring the support. The administrative complexity does not adjust for caregiver load. The bureaucratic shorthand does not soften when the household is in crisis. Families drop out of programs they were eligible for, or never apply at all.¹⁴

The Structural Contradiction

The integrated effect of the documented pattern is a structural contradiction that can be stated precisely. The Government of Alberta determined, through its own formal adjudication process, that 79,290 people have severe and permanent disabilities preventing them from earning a livelihood. The defining clinical features of the largest groups in that population — cognitive disorders, mental illness — include difficulty with sustained executive function, difficulty processing complex written communication, difficulty initiating and completing multi-step administrative tasks, and difficulty navigating institutional systems without external support. The same Government then designed the compliance, consultation, and reapplication architecture under which the same population is required to read forms at Grade 12 to 16 reading level, navigate multi-step bureaucratic processes across multiple agencies, meet deadlines without flexibility for medical flares or hospitalizations, and respond to consultation invitations whose subject matter is the criteria that will subsequently be applied to determine their continued eligibility — criteria the Government has not published.¹⁵

The contradiction is not incidental. It is the operating structure of the program. The recipient population was assessed and approved into AISH because of the limitations that make the compliance system inaccessible. The same limitations are then operationally treated, by the program's communications and processes, as if they were not present. A population whose qualifying conditions include difficulty with multi-step administrative tasks is required to perform multi-step administrative tasks in order to keep the support that recognized the difficulty in the first place.

The community response to the documented pattern is itself a data point. The Alberta AISH recipient community group tripled in size over the period from spring 2025 to spring 2026, from approximately two thousand members to more than six thousand, according to the group's administrator. Community groups of this kind do not triple in size when recipients feel supported and informed. They triple when recipients feel abandoned and afraid, and when they are actively seeking information and community that the formal system has ceased to provide. The growth of informal recipient-led networks during the same period the formal supports were being removed is not coincidence. It is the predictable consequence of the removal.¹⁶

The Constructive Knowledge Doctrine

A principle in administrative law and in human rights jurisprudence operates across the situation this chapter documents. Where an institution knows, or where in the circumstances it ought to know, that a standard it is applying cannot be met by the population to whom it is applied, the institution's continued application of the standard is not procedurally innocent. The constructive-knowledge doctrine attaches the institution's knowledge to its conduct regardless of whether each individual decision-maker subjectively intends the foreseeable result.¹⁷

The Government of Alberta is on documented constructive notice of every element examined in this chapter. The 79,290 caseload figure and the breakdown by primary medical condition are from its own open data. The Grade 12 to 16 reading level of its forms is a function of its own document drafting practice. The non-publication of the Medical Review Panel adjudication standards is its own administrative choice. The Ministerial Order setting the employment income exemption calculation is within its own authority to issue or withhold. The 24 percent DTC completion rate is documented in federal reports the Government has the operational capacity to consult. The reading-level standards in federal Treasury Board guidance, in disability-advocacy practice, and in the Convention itself are public and accessible.

The Government has been formally notified of the pattern documented here through Inclusion Alberta's public analyses, through the campaign's evidentiary reports published and distributed to government addressees, through municipal council motions, through the November 26, 2025 Inclusion Alberta media release, through the MS Canada statement of November 28, 2025, through Alberta Medical Association publications including the November/December 2025 issue of *Alberta Doctors' Digest*, and through direct ministerial correspondence from named individual recipients. The constructive knowledge condition is met as a matter of documented record.¹⁸

Where the constructive knowledge condition is met, the continued operation of the inaccessible standard is not an administrative failure subject to incremental reform. It is the operating design of the program. The information-asymmetry condition is, on the documented record, sustained because it produces the outcomes the surrounding fiscal architecture serves. The recipient who cannot complete the reapplication does not remain on AISH. The applicant who cannot navigate the DTC chain does not access the CDB. The family that cannot parse the FSCD documentation does not receive the support. The caseload reduces by attrition. The fiscal saving is the saving the policy framework was structured to produce.

What This Chapter Establishes

Four observations follow.

First, the CRPD Article 4(3) consultation standard examined in Chapter IV cannot meaningfully operate where the upstream information condition is not met. A population that has not been informed of the operational rules of the program being designed for them, of the criteria against which their continued eligibility will be assessed, or of the standards by which their documentation will be evaluated, cannot meaningfully consent to the program through any consultation mechanism the consulting authority designs.

Second, the documented information condition in Alberta as of the consultation window in September 2025 and as of the transition window in July 2026 fails the standards CRPD Articles 9 and 21 establish, the standards the federal *Accessible Canada Act* articulates, and the standards the *Alberta Human Rights Act* Section 4 duty of accommodation triggers. The failure is not incidental. It is documented in the reading-level of the forms, in the non-publication of the adjudication standards, in the deferral of the Ministerial Order, and in the structural contradiction between the qualifying conditions and the navigational demands.

Third, the structural contradiction is precise. The population the program was designed to serve was assessed and approved because of the limitations that make the program's communications and processes inaccessible. The same limitations are then operationally treated as if they were not present. The diagnostic criteria on which approval was granted are the diagnostic criteria the maintenance system then requires the recipient to override.

Fourth, the constructive knowledge doctrine is engaged on the documented record. The Government of Alberta is on notice, through its own open data, through federal accessibility jurisprudence, through formal correspondence from civil society organizations, and through municipal advocacy, of every element of the pattern this chapter documents. The continued operation of the inaccessible architecture, in the face of the documented pattern, is not an administrative failure capable of incremental reform without a change in the institutional position. It is the operating structure that produces the fiscal outcomes the surrounding policy framework was designed to produce.

The chapter that follows examines what is being built in the institutional space the consultation standard would have constrained — the architecture currently under construction in Alberta and in jurisdictions adjacent to it. The information condition documented in this chapter is the condition under which that construction has been authorized to proceed.

Notes — Chapter V

1. *Convention on the Rights of Persons with Disabilities*, Articles 9 (Accessibility) and 21 (Freedom of Expression and Access to Information); Canada ratified March 11, 2010. The full text of both articles is available at the UN Department of Economic and Social Affairs, Division for Inclusive Social Development, <https://www.un.org/development/desa/disabilities/>.
2. *Accessible Canada Act*, SC 2019, c 10; Bill C-81, *An Act to Ensure a Barrier-Free Canada*, tabled June 20, 2018 by the Honourable Carla Qualtrough, then Minister of Public Services and Procurement and Accessibility; Royal Assent June 21, 2019; in force July 11, 2019. Statutory objective: realization of a Canada without barriers by January 1, 2040.
3. *Alberta Human Rights Act*, RSA 2000, c A-25.5, section 4; *Canadian Charter of Rights and Freedoms*, sections 7 and 15, Part I of the *Constitution Act, 1982*; *Eldridge v. British Columbia (Attorney General)*, [1997] 3 SCR 624 (substantive equality requires accommodation rather than merely identical text on paper).
4. Government of Canada, Treasury Board Secretariat, Communications Policy of the Government of Canada and associated plain-language guidance; Plain Language Action and Information Network (PLAIN), plainlanguage.gov; Statistics Canada, Programme for the International Assessment of Adult Competencies (PIAAC), Adult Literacy and Life Skills Survey data.
5. The Alberta Disability System Breakdown, *The Access Gap: Penalized for Non-Ability*, May 2026, Section 5 (Bureaucratic Legalese as Gatekeeping). Independent readability analysis using standard tools (Flesch-Kincaid Grade Level) of Government of Alberta AISH and ADAP documentation and federal CDB application materials.
6. Government of Alberta, AISH Caseload Open Data, September 2025, open.alberta.ca; The Alberta Disability System Breakdown, *What Disability Actually Means*, May 2026 (plain-language guide documenting the caseload breakdown by primary medical condition).
7. Government of Alberta, family physician availability data; The Alberta Disability System Breakdown, *The Access Gap: Penalized for Non-Ability*, May 2026, Section 1 (The Structural Contradiction).
8. The Alberta Disability System Breakdown, *AISH Evidentiary Report Series*, April 2026, Section 6 (The Questions That Require Answers Before July 1, 2026); *Compound Failure Brief*, April 2026, Pillar III(d) (the Removal of Appeal Rights and the Medical Review Panel architecture).
9. Minister Jason Nixon, Minister of Assisted Living and Social Services, letter to Genevieve Bissonette, April 3, 2026 (confirming the Ministerial Order on employment income calculations would be issued “later this spring”); The Alberta Disability System Breakdown, *AISH Evidentiary Report Series*, April 2026, on the deferred Ministerial Order.
10. The Alberta Disability System Breakdown, *Compound Failure Brief*, April 2026, Pillar III(d), documenting the specific categories of adjudication criteria that have not been published in any form a prospective applicant could review before the post-transition Medical Review Panel adjudication.
11. The Alberta Disability System Breakdown, *Compound Failure Brief*, April 2026, Pillar III(d), analysis of the exemption-category structure and the operative “framing-amenable” criterion derived from it.
12. Government of Canada Disability Advisory Committee, 2024 report; documented 24 percent completion rate for online DTC applications and 76 percent abandonment rate. DTC physician fee structure documented in The Alberta Disability System Breakdown, *Access Barrier Report*, April 2026, with primary-source receipts and correspondence.
13. The Alberta Disability System Breakdown, *Access Barrier Report*, April 2026, documenting the federal-provincial DTC/CDB/AISH chain through timestamped correspondence in a primary-source case file spanning 2019 to 2026.
14. The Alberta Disability System Breakdown, *The Access Gap: Penalized for Non-Ability*, May 2026, Section 5.3 (The FSCD/PDD Form Comparison). The pattern of caregiver-mediated administrative complexity inheriting the same readability and accessibility issues as the recipient-mediated AISH/ADAP architecture.

15. See note 7 and accompanying text; The Alberta Disability System Breakdown, *Information Asymmetry Plain Language Companion*, April 2026, naming the structural contradiction at the policy-design level.
16. Community group growth documented in the campaign’s observations of the Alberta AISH recipient community group — the same kind of network that emerged in disability communities historically when formal support structures failed; see The Alberta Disability System Breakdown, *Indigenous Disability Alberta Collection*, May 2026, for the parallel pattern in Indigenous disability communities during the same period.
17. The constructive-knowledge doctrine operates across multiple branches of Canadian and international law, including administrative-law jurisprudence on procedural fairness, human-rights jurisprudence on the duty to accommodate (see, e.g., *British Columbia (Public Service Employee Relations Commission) v. BCGSEU*, [1999] 3 SCR 3 (the *Meiorin* standard)), and tort jurisprudence on knowing or willful continuation of harmful conduct. The doctrine’s application to the disability-policy context is developed in The Alberta Disability System Breakdown, *Charter and CRPD Analysis*, April 2026.
18. Inclusion Alberta, “Bill 12 erodes rights and deepens poverty for Albertans with disabilities,” media release, November 26, 2025; MS Canada, November 28, 2025; Alberta Doctors’ Digest, “Pushing back against changes that make life harder for disabled Albertans,” November/December 2025; The Alberta Disability System Breakdown distribution record covering Tier 1 (provincial, federal, oversight, municipal), Tier 2 (coalition), and Tier 3 (media) recipients, 2025–2026.

Chapter VI

Modern Eugenics Momentum

The Institutional Rebuild

What Is Being Built

Across the period 2024 to 2026, a cluster of legislative and executive measures has expanded the legal authority of governments in Canada and the United States to detain and to compel treatment of persons with severe substance-use, mental health, and concurrent disorders. The measures are not identical. Each is responding to a different combination of opioid mortality, encampment crisis, and political pressure. The legal frameworks they create differ in their procedural safeguards, their authorizing officials, and their scope. The pattern, taken together, is consistent: legislative authority to compel treatment is expanding rapidly in jurisdictions that are, in the same period, constraining the community-based supports through which voluntary care has historically been delivered.

This chapter documents what has been built. Five jurisdictions are examined: Alberta, Saskatchewan, British Columbia, Ontario, and the United States at the federal level. The chapter then applies the framework Chapter II established — the structural-eugenics analysis — to the documented pattern. The structural argument is the effect argument. It is not the claim that contemporary governments harbour the explicit eugenic intent of the early twentieth century. It is the claim that the pattern of building large secure-treatment infrastructure for predominantly poor, disabled, and disproportionately Indigenous populations, in the absence of equivalent investment in community-based alternatives, is structurally continuous with the institutionalization pattern documented in Chapter I — and that the documented overlap of the affected populations with the populations historically subject to the *Sexual Sterilization Act* is not incidental.

Alberta: The Compassionate Intervention Act

On April 15, 2025, the Government of Alberta introduced Bill 53, the *Compassionate Intervention Act*. The Act received Royal Assent on May 15, 2025. It establishes a framework for involuntary treatment of persons with severe substance-use or addiction whose conduct is determined to be likely to cause substantial harm to themselves or others within a reasonable time.¹

The Act's operative provisions authorize adult family members, guardians, healthcare professionals, police officers, and peace officers to apply for an assessment order. A three-person Compassionate Intervention Commission, composed of a lawyer, a physician, and a member of the public, all appointed by the provincial government, reviews applications and issues legally binding treatment decisions. The Act provides for a 72-hour assessment and detoxification period during which a person may be detained

for assessment, with hearings before the full Commission required within that window. Secure treatment orders of up to three months may be issued; community-based treatment orders may extend to six months. Care-plan reviews occur every three weeks during the treatment period. The Act replaces the 2006 *Protection of Children Abusing Drugs Act* (PChAD) for youth.²

The Act expands the provincial involuntary-treatment framework beyond the existing *Mental Health Act*, RSA 2000, c M-13. The *Mental Health Act* framework operates against persons with mental disorders meeting an impaired-capacity test. The *Compassionate Intervention Act* operates against persons with severe substance-use or addiction even where the capacity test is met. The substantive evidentiary threshold for involuntary detention and treatment under the new framework is lower than the threshold the existing *Mental Health Act* infrastructure required.³

The infrastructure investment associated with the Act has been progressively announced. Budget 2025 allocated \$180 million over three years; Budget 2026 increased the allocation to \$291 million over three years (2026–2029) for the construction of two 150-bed compassionate-intervention centres in Edmonton and Calgary. Construction is to begin in 2026 with the centres expected to open by 2029. Approximately 100 secure beds are being established in existing facilities during the implementation phase. Recovery Alberta operates as the provincial health agency administering the framework.⁴

The professional-and-academic response has been substantial. Eric Adams, a constitutional law professor at the University of Alberta, has stated that the legislation is expected to face Charter challenge once it becomes operational, given its interference with Charter Section 7 rights (life, liberty, security of the person) and Section 9 rights (freedom from arbitrary detention). The Canadian Bar Association — Alberta Branch, in a May 9, 2025 letter, raised concerns including the substantive overlap with existing legislation (the *Personal Directives Act*, the *Adult Guardianship and Trusteeship Act*, the *Youth Criminal Justice Act*), procedural-fairness questions including the ability of the “client” to participate in the assessment process, presumption-of-capacity questions, and consent-and-treatment provisions including authorization of medication administration without consent.⁵

The *Canadian Medical Association Journal* published an ethical analysis applying the Kass framework for public-health interventions to the Act, concluding that the framework presents substantive risks across all three Kass-framework domains — privacy, liberty, and justice — with operational consequences including the potential for disproportionate targeting of structurally marginalized populations including First Nations, Inuit, and Métis individuals. The Alberta Medical Association, in a May 7, 2025 statement, identified operational concerns including post-discharge care, legal safeguards, and the operational interaction with social factors including housing and income. Field Law, a regulatory law firm advising health professionals, published a January 6, 2026 analysis documenting operational tensions between the new framework and the existing *Mental Health Act* infrastructure.⁶

The opioid-crisis context within which the Act has been developed is severe. Alberta documented more than 1,800 opioid-related deaths in 2023 and 1,189 deaths in 2024. The March 2025 Edmonton-zone single-month record reached 87 deaths. Carfentanil involvement reached 68 percent of provincial overdoses and 78 percent of Edmonton overdoses in the January to May 2025 period. Opioid Dependency Program enrollment grew from 3,288 clients in 2018 to 12,299 clients in 2024. The provincial policy

direction during the same period has shifted from harm reduction toward recovery-focused approaches, with supervised-consumption-site closures across the province.⁷

Saskatchewan: A Parallel Statute

On December 5, 2025, Premier Scott Moe’s government in Saskatchewan introduced legislation under the same name as Alberta’s: the *Compassionate Intervention Act*. The Saskatchewan bill was tabled on the final day of the fall legislative sitting. The legislation passed on May 5, 2026. The Act is expected to come into force in fall 2026 after regulations are finalized.⁸

The Saskatchewan framework follows the Alberta architecture in substantive structure. Family members may request treatment through the court system; law enforcement may intervene where substance use puts life at serious risk; medical professionals may refer. The first Compassionate Intervention Assessment Centre will be located in North Battleford. The involuntary inpatient unit will be situated at Saskatchewan Hospital North Battleford, an existing psychiatric facility. The Government of Saskatchewan reports that approximately 150 stakeholders participated in information sessions during the legislation’s development.⁹

The opposition record across the Saskatchewan disability, medical, and civil-rights sector mirrors the Alberta record. The Saskatchewan Medical Association issued a joint statement with other physician organizations identifying capacity concerns: “The healthcare system is not ready for this. Emergency departments and other frontline services are already under severe strain, and the system does not have the capacity required to safely implement involuntary pathways.” The John Howard Society of Saskatchewan, which advocates for incarcerated people, echoed the physicians’ concerns. The Opposition NDP voted against the bill, with leader Carla Beck arguing the province had not put adequate protections in place.¹⁰

The Government cited support from Saskatoon Tribal Chief Mark Arcand, who noted the framework’s stated focus on culturally responsive care. The endorsement is, on the documented record, one of relatively few from Indigenous leadership; the broader Indigenous-disability and First-Nations health-policy sector has not, in the documented public record, endorsed the framework.¹¹

British Columbia: Mental Health Act Expansion

British Columbia has expanded involuntary care under its existing *Mental Health Act* framework rather than through a separate Compassionate Intervention Act. On September 15, 2024, Premier David Eby announced a secure-care framework targeting people with concurrent addiction, mental illness, and acquired brain injuries from toxic-drug poisonings. The provincial framework, on the documented public record, is the most direct response to a provincial overdose crisis that has produced more than 15,000 deaths since 2016, when the province declared a public health emergency.¹²

In summer 2024, the province appointed Dr. Daniel Vigo as British Columbia’s first chief scientific adviser for psychiatry, toxic drugs and concurrent disorders. On March 12, 2025, Dr. Vigo issued clinical guidance specifying that involuntary care under the *Mental Health Act* cannot be used to prevent harmful “risk-taking” by people who use drugs whose behaviour is not related to mental impairment. The guidance

attempts to articulate the boundary between the legitimate medical application of involuntary care for persons whose decision-making capacity is impaired by severe mental illness and the use of involuntary care as a policy response to public-space disorder. The boundary is, in the documented operational practice that has followed, subject to substantive interpretive variation across health authorities and facilities.¹³

The first correctional involuntary-care facility, a 10-bed unit at the Surrey Pretrial Services Centre, opened on April 24, 2025. The first secure housing-and-care facility, Alouette Homes on the grounds of the Alouette Correctional Centre in Maple Ridge, opened in June 2025 with 18 long-term involuntary care beds. More than 400 mental-health beds are being built or modernized across the province — approximately 280 outdated beds modernized and more than 140 new beds added — under the framework. On November 24, 2025, the province introduced *Mental Health Act* amendments operationalizing the framework expansion.¹⁴

The framework targets, in the language of provincial communications, people with “concurrent mental-health and substance-use challenges, and brain injuries from toxic-drug poisonings.” The triple-condition profile the framework engages overlaps substantively with the documentation in the British Columbia Centre on Substance Use literature on the population most affected by the toxic drug supply — a population the same Centre has identified as inadequately served by the involuntary-care expansion in the absence of comparable expansion in voluntary community-based services.¹⁵

Ontario: The Encampment Frame

Ontario has not, as of the date of this chapter, passed compassionate-intervention legislation parallel to Alberta’s or Saskatchewan’s. The Ontario framework is moving more slowly, but along a recognizable trajectory. In May 2025, the province announced a study of involuntary addictions treatment for people in the correctional system — in jail, on probation, on parole. The Ontario approach explicitly cites the British Columbia model.¹⁶

The Ontario opioid context is, in absolute terms, the largest in Canada. The province documented 2,600 or more opioid-related deaths in 2023, a 50 percent increase over 2019. Approximately 48 percent of inmates carry substance-use alerts on their profiles (February 2024 data); approximately 28 percent of inmates are voluntarily on prescribed opioid agonist therapy. The Canadian Mental Health Association — Ontario has identified 1,400 or more encampments across the province and an average of seven deaths per day from drug poisonings, with \$18 billion in 2020 social costs from substance use.¹⁷

The political pressure for expansion has come substantively from municipal leadership. In October 2024, Ontario Big City Mayors — an organization representing 29 mayors of cities over 100,000 population — collectively asked the province to review mental-health laws and the scope of involuntary treatment. Mayor Alex Nuttall of Barrie and Mayor Cam Guthrie of Guelph have been particularly outspoken. The CMHA Ontario position, by contrast, has been to oppose involuntary-treatment expansion and to ask for \$113 million in the 2025-26 budget for community mental health, including 5,000 additional supportive housing units. The expansion of involuntary treatment infrastructure and the expansion of community supportive-housing infrastructure are being requested by different actors at the same time. The province

has, on the documented record, moved more quickly on the first than on the second.¹⁸

Ontario does have an existing Bill 53, the *Dignity and Mental Health in Jails Act, 2025*, distinct from Alberta's Bill 53. The Ontario Bill 53 addresses correctional mental-health units, requires that 20 percent of inmate beds in correctional institutions be in mental-health support units, and prohibits cruel, inhumane, or degrading treatment within corrections. It is not, on its face, an involuntary-treatment expansion in the sense documented in Alberta and Saskatchewan, but it operates in the same conceptual space — the provision of mental-health treatment within carceral infrastructure.¹⁹

The United States: Executive Order 14321

On July 24, 2025, President Donald J. Trump signed Executive Order 14321, titled “Ending Crime and Disorder on America’s Streets.” The Order, which adopts pledges from the Project 2025 blueprint, expands the use of involuntary civil commitments for adults experiencing homelessness and serious mental illness, directs the Department of Justice to evaluate homeless individuals arrested for federal crimes as “sexually dangerous persons,” and ends federal funding for Housing First programs. It directs the Secretary of Health and Human Services and the Secretary of Housing and Urban Development to provide federal grants and legal assistance to state and local governments for “maximally flexible civil commitment, institutional treatment, and step-down treatment standards.”²⁰

The legal context is the 1999 United States Supreme Court decision *Olmstead v. L.C.*, 527 U.S. 581 — a twenty-six-year-settled precedent guaranteeing persons with disabilities the right to live in community with necessary support. The Executive Order, in the analysis of the National Alliance to End Homelessness, “defies a 26-year settled legal precedent in *Olmstead v. L.C.*” The 2024 Supreme Court decision in *Grants Pass v. Johnson*, which permits ticketing, fining, and arresting people sleeping on public property, provides the procedural environment within which the Executive Order’s institutionalization framework is being deployed.²¹

The opposition to the Executive Order has been broad. The National Alliance to End Homelessness, the Kaiser Family Foundation in an August 20, 2025 analysis, the law firm Epstein Becker Green in a July 30, 2025 legal review, the Prison Policy Initiative in an August 5, 2025 analysis, and Shannon Minter of the National Center for LGBTQ Rights have each identified substantive concerns. The Epstein Becker Green analysis observes that “the Executive Order does not include access to federal funds to build out treatment capacity for people who are civilly committed” and that “the Trump administration’s efforts so far have involved cutting funds for mental health treatment, not offering to fund increased capacity.”²²

The Hari/Rat Park Frame Reapplied

The framework Chapter II established turns on the distinction between policy responses to addiction that operate through isolation and policy responses that operate through connection. The Rat Park studies conducted by Bruce K. Alexander and colleagues at Simon Fraser University in the 1970s established experimentally what the public-health literature has subsequently confirmed: isolated, deprived environments produce predictable adaptive responses, including substance dependence, that are mispathologized as individual failure when the structural conditions producing them are not also examined. The work has been synthesized for general audiences most prominently by Johann Hari, whose central reframe is that the opposite of addiction is not sobriety but connection.²³

The Portugal decriminalization model, introduced in July 2001, is generally cited as the structural opposite of the mandate-treatment framework. Portugal's framework decriminalizes personal possession of small quantities of drugs and routes persons identified with problematic substance use into voluntary treatment, integrated with housing, employment, and community-reintegration supports. The documented outcomes, across European Monitoring Centre data and the academic literature, include substantial reductions in HIV transmission, overdose deaths, and the incarceration of drug users, alongside documented increases in treatment uptake.²⁴

The framework currently being built across Alberta, Saskatchewan, British Columbia, Ontario, and at the United States federal level operates on the opposite premise. The mechanism it deploys is mandatory treatment within secure facilities, often co-located with or adjacent to correctional infrastructure (Surrey Pretrial Services Centre; Alouette Correctional Centre grounds; the correctional-system pathway in Ontario; the Saskatchewan Hospital psychiatric facility), authorized under legal frameworks that lower the evidentiary threshold below the existing mental-health-act standards. The pattern occurs in the same jurisdictions and during the same period in which voluntary community-based supports are being constrained — through supervised-consumption-site closures, harm-reduction funding reductions, the reorientation of addictions services toward abstinence-only models, and (in the United States) the federal defunding of Housing First programs.²⁵

The structural argument is direct. A government that responds to a population's isolation by building larger isolation infrastructure, while simultaneously dismantling the community-based mechanisms through which that population's connection could be supported, has chosen the policy response the Rat Park literature identifies as the response that produces the pattern the policy is presented as addressing. The mechanism is recursive. The infrastructure expansion does not address the conditions producing the crisis; it expands the institutional capacity to manage the population the crisis has produced.

The Parallel to the Provincial Training School Period

Chapter I documented the institutional infrastructure of the Alberta eugenic period: the Provincial Training School at Red Deer, the Alberta Hospital network including Ponoka, Oliver, and Edmonton, the Michener Centre, and the auxiliary facilities through which persons identified as “feeble-minded,” “psychopathic,” or “mentally defective” were institutionalized and, in approximately 2,800 documented cases, sterilized under the authority of the Eugenics Board. The infrastructure was not built on explicit eugenic legislation alone. It was built on a broader consensus that institutional confinement was the appropriate response to populations the surrounding social and economic structure could not or would not accommodate.²⁶

The populations the historical institutionalization framework engaged are substantively the same populations the contemporary involuntary-treatment frameworks engage. The overlap is documented across multiple dimensions. The Alberta *Sexual Sterilization Act* period’s disproportionate impact on Indigenous women is paralleled by the CMAJ ethical analysis warning of disproportionate targeting of First Nations, Inuit, and Métis individuals under the *Compassionate Intervention Act*. The historical institutionalization of persons with cognitive disability and mental illness is paralleled by the contemporary framework’s targeting of persons with concurrent mental-health and substance-use conditions — conditions whose presentation frequently overlaps with cognitive disability and acquired brain injury. The historical institutionalization of poor persons whose poverty was characterized as moral failure is paralleled by the contemporary framework’s targeting of unhoused persons whose homelessness is characterized in the United States Executive Order as “crime and disorder.”²⁷

The mechanism of authorization differs. The Eugenics Board authorized sterilization. The contemporary Commissions and review boards authorize involuntary treatment and detention in secure facilities. Sterilization is not a contemporary operational tool of the framework. Detention and compelled treatment in secure facilities are.

The structural argument is the effect argument, not the intent argument. The framework that is being built does not require the eugenic intent of the 1928 legislation to produce structurally continuous outcomes for structurally continuous populations. It requires only the operating premise that some populations are properly subjects of institutional management rather than community participants — and the operational capacity to act on that premise at scale.

What This Chapter Establishes

Five observations follow.

First, a coordinated expansion of involuntary-treatment legal authority is occurring across multiple Canadian provinces and at the United States federal level during the period 2024 to 2026. The frameworks differ in detail. The pattern, taken together, is consistent.

Second, the expansion is occurring in the same jurisdictions and during the same period in which community-based supports for the same population are being constrained. The Alberta Compassionate

Intervention Act has proceeded alongside supervised-consumption-site closures and the Bill 12 reduction of disability income documented in Chapter IV. The United States Executive Order has proceeded alongside the federal defunding of Housing First. The Saskatchewan and Ontario frameworks are proceeding in the absence of comparable expansion in voluntary community-based services. The pattern is not a single jurisdictional choice. It is a cross-jurisdictional policy direction.

Third, the populations the contemporary frameworks engage overlap substantively with the populations the historical institutionalization framework engaged. Persons with concurrent mental-health and substance-use conditions, persons with acquired brain injuries, persons experiencing homelessness, and First Nations, Inuit, and Métis individuals appear disproportionately across the documented analyses of both periods. The overlap is not incidental. It is the documented structural feature of the policy choice.

Fourth, the framework operates against the public-health evidence on the conditions producing the crisis the framework is presented as addressing. The Rat Park research tradition, the Portugal decriminalization outcome data, the Housing First evidence base, and the harm-reduction literature each document that connection-based and community-based responses produce better outcomes for the affected populations than isolation-based and institutional responses. The framework currently being built operates in the opposite direction.

Fifth, the historical parallel is direct. The Provincial Training School period in Alberta, the parallel institutionalization patterns documented in adjacent jurisdictions, and the contemporary involuntary-treatment expansion all share the operating premise that some populations are properly subjects of institutional management rather than community participants. The mechanism of authorization has changed. The operating premise has not.

The chapter that follows develops the closing argument of the piece. It examines, across all six chapters preceding it, the question the piece's title puts on the record: whether the consent the contemporary frameworks claim to have obtained from the populations they engage was ever the consent the standard requires, or whether consent was the narrative offered to obscure a policy direction that does not, on the documented evidence, require consent to proceed.

Notes — Chapter VI

1. Legislative Assembly of Alberta, Bill 53, *Compassionate Intervention Act*, 31st Legislature, First Session; introduced April 15, 2025; Royal Assent May 15, 2025. Government of Alberta, “Compassionate Intervention,” alberta.ca/compassionate-intervention.
2. *Compassionate Intervention Act*, statutory provisions on assessment-order authority, Commission composition, 72-hour assessment-and-detoxification period, secure-treatment-order duration (up to 3 months), community-based-treatment-order duration (up to 6 months), and care-plan review intervals (every 3 weeks); CBC News, “Alberta introduces controversial involuntary addictions treatment bill,” April 15, 2025, cbc.ca/news/canada/edmonton/alberta-introduces-controversial-involuntary-addictions-treatment-bill-1.7511051.
3. *Mental Health Act*, RSA 2000, c M-13; *Compassionate Intervention Act*, evidentiary-threshold and scope provisions; Field Law, “Compassion vs. Coercion: Regulatory Impacts of Bill 53,” January 6, 2026, fieldlaw.com/insights/publication/Compassion-vs-Coercion-Regulatory-Impacts-of-Bill-53.
4. Government of Alberta, Budget 2025 and Budget 2026, infrastructure allocations for compassionate-intervention centres; Recovery Alberta program documentation; The Alberta Disability System Breakdown, *What Dropped This Week* (Plain Language Edition), May 2026.
5. Statements of Eric Adams (constitutional law, University of Alberta), documented in contemporaneous Canadian legal and media coverage; Canadian Bar Association — Alberta Branch, “Letter re Compassionate Intervention Act,” May 9, 2025, cba-alberta.org/our-impact/submissions/may-2025-letter-re-compassionate-intervention-act/.
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7. Government of Alberta opioid surveillance data, 2023 and 2024 calendar years; The Alberta Disability System Breakdown, *Building Up*, Part One, Section 1.7 (Mental Health and Addiction as a Lifecycle Dimension), May 2026; Global News, “This Isn’t Over Yet: Opioid-Related Deaths Reach New Heights in Edmonton,” July 3, 2025.
8. Government of Saskatchewan, news release, “Legislation Passes for Compassionate Intervention for Addictions Treatment,” May 5, 2026, saskatchewan.ca/government/news-and-media/2026/may/05/; CBC News, “Sask. introduces involuntary treatment legislation as fall sitting ends,” December 5, 2025; Global News, “Saskatchewan passes law on involuntary drug treatment,” May 2026.
9. Government of Saskatchewan news release, May 5, 2026; Meridian Source, “New Saskatchewan law would allow compulsory addiction care,” December 8, 2025; CBC News, “Involuntary addiction treatment now legal in Sask. despite medical groups’ warnings,” May 6, 2026.
10. Saskatchewan Medical Association, joint statement with other physician organizations on the Compassionate Intervention Act, 2025-2026; John Howard Society of Saskatchewan, public statements opposing the framework; Saskatchewan NDP, opposition statements documented through legislative record and contemporaneous media coverage.
11. Meridian Source, “New Saskatchewan law would allow compulsory addiction care,” December 8, 2025, reporting the statement of Saskatoon Tribal Chief Mark Arcand.
12. Government of British Columbia, news release, “Province launches secure care for people with brain injury, mental illness, severe addiction,” September 15, 2024, news.gov.bc.ca/releases/2024PREM0043-001532; The Globe and Mail, “B.C. to introduce involuntary care for people with concurrent addiction, mental disorders, premier says,” September 15, 2024.

13. Government of British Columbia, news release, “B.C. improving care for people with mental-health, substance-use challenges,” March 12, 2025, news.gov.bc.ca/releases/2025HLTH0015-000202; CBC News, “B.C. doctors get new guidance on involuntary care for drug users,” March 12, 2025.
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26. See Chapter I and its associated endnotes for the full record of the Alberta Provincial Training School and the broader institutional infrastructure of the Sexual Sterilization Act period.
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